

Medical Group Participation Agreement

This Agreement is entered into by and between UnitedHealthcare Insurance Company, UnitedHealthcare of Washington, Inc., PacifiCare Life Assurance Company, and PacifiCare Life and Health Insurance Company ("Carrier") contracting on behalf of each entity individually, and RADIANTCARE RADIATION ONCOLOGY LLC ("Medical Group").

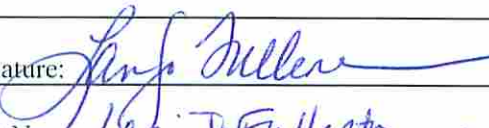
This Agreement is effective on the later of the following dates (the "Effective Date"):

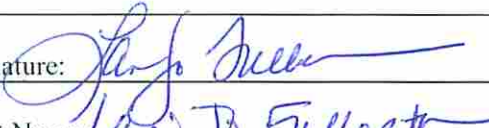
- i) or
- ii) the first day of the first calendar month that begins at least 30 days after the date when this Agreement has been executed by all parties.

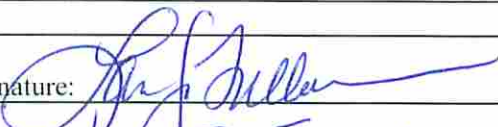
Through contracts with physicians and other providers of health care services, Carrier maintains one or more networks of providers that are available to Customers. Medical Group is a provider of health care services.

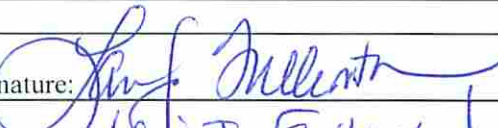
Carrier wishes to arrange to make Medical Group's services available to Customers. Medical Group wishes to provide such services, under the terms and conditions set forth in this Agreement.

The parties therefore enter into this Agreement.

UnitedHealthcare Insurance Company	Medical Group: RADIANTCARE RADIATION ONCOLOGY LLC
Street 7525 SE 24TH ST STE 200	Street 4525 3RD AVE SE STE 100
City MERCER ISLAND	City LACEY
State WA	State WA Zip Code 98503-1010
Zip Code 98040	NCST_0046600_002417609_MGA2012
Signature:	Signature: 
Print Name	Print Name Lani Jo Fullerton
Title VP NETWORK MANAGEMENT	Title Director of Strategic & Financial Planning
Date	Date 1/15/13

PacifiCare Life and Health Insurance Company	Medical Group: RADIANTCARE RADIATION ONCOLOGY LLC
Street 7525 SE 24TH ST STE 200	Street 4525 3RD AVE SE STE 100
City MERCER ISLAND	City LACEY
State WA	State WA Zip Code 98503-1010
Zip Code 98040	
Signature:	Signature: 
Print Name DUSTIN TAYLOR	Print Name Lani Jo Fullerton
Title VP NETWORK MANAGEMENT	Title Director of Strategic & Financial Planning
Date	Date 1/15/13

PacifiCare Life Assurance Company	Medical Group: RADIANTCARE RADIATION ONCOLOGY LLC
Street 7525 SE 24TH ST STE 200	Street 4525 3RD AVE SE STE 100
City MERCER ISLAND	City LACEY
State WA	State WA Zip Code 98503-1010
Zip Code 98040	
Signature:	Signature: 
Print Name DUSTIN TAYLOR	Print Name Laci Jo Fullerton
Title VP NETWORK MANAGEMENT	Title Director of Strategic & Financial Planning
Date	Date 1/15/13

UnitedHealthcare of Washington, Inc.	Medical Group: RADIANTCARE RADIATION ONCOLOGY LLC
Street 7525 SE 24TH ST STE 200	Street 4525 3RD AVE SE STE 100
City MERCER ISLAND	City LACEY
State WA	State WA Zip Code 98503-1010
Zip Code 98040	
Signature:	Signature: 
Print Name DUSTIN TAYLOR	Print Name Laci Jo Fullerton
Title VP NETWORK MANAGEMENT	Title Director of Strategic & Financial Planning
Date	Date 1/15/13

IN THE EVENT THIS AGREEMENT INCLUDES AN ADDITIONAL SIGNATURE BLOCK BELOW THIS STATEMENT FOR CARRIER, THIS AGREEMENT IS NOT BINDING UPON CARRIER UNLESS BOTH OF THE TWO CARRIER SIGNATURE BLOCKS ARE EXECUTED.

Article I.
Definitions

The following terms when used in this Agreement have the meanings set forth below:

- 1.1 **"Benefit Plan"** means a certificate of coverage, summary plan description, or other document or agreement, whether delivered in paper, electronic, or other format, under which a Payer is obligated to provide coverage of Covered Services for a Customer.
- 1.2 **"Covered Service"** is a health care service or product for which a Customer is entitled to receive coverage from a Payer, pursuant to the terms of the Customer's Benefit Plan with that Payer.
- 1.3 **"Customary Charge"** is the fee for health care services charged by Medical Group that does not exceed the fee Medical Group would ordinarily charge another person regardless of whether the person is a Customer.
- 1.4 **"Customer"** is a person eligible and enrolled to receive coverage from a Payer for Covered Services.
- 1.5 **"Medical Group Physician"** is a Doctor of Medicine ("M.D."), or a Doctor of Osteopathy ("D.O."), duly licensed and qualified under the laws of the jurisdiction in which Covered Services are provided, who practices as a shareholder, partner, or employee of Medical Group, or who practices as a subcontractor of Medical Group. However, a subcontractor of Medical Group is a Medical Group Physician only with regard to services rendered to patients of Medical Group and billed under Medical Group's tax identification number. Additionally, a subcontractor is not a Medical Group Physician with regard to any services rendered in a physician's office other than those locations listed in Appendix 1.
- 1.6 **"Medical Group Non-Physician Provider"** is a surgical assistant, physician assistant, nurse practitioner, physical therapist, occupational therapist, speech therapist, mental health provider, or licensed social worker, who is duly authorized under the laws of the jurisdiction in which Covered Services are provided, and who renders Covered Services as an employee or subcontractor of Medical Group. However, a subcontractor of Medical Group is a Medical Group Non-Physician Provider only with regard to services rendered to patients of Medical Group and billed under Medical Group's tax identification number. Additionally, a subcontractor is not a Medical Group Non-Physician Provider with regard to any services rendered in a physician's office other than those locations listed in Appendix 1.
- 1.7 **"Medical Group Professional"** is a Medical Group Physician or a Medical Group Non-Physician Provider.
- 1.8 **"Payment Policies"** are the guidelines adopted by Carrier for calculating payment of claims under this Agreement. The Payment Policies may change from time to time as discussed in section 7.4 of this Agreement.
- 1.9 **"Payer"** is an entity obligated to a Customer to provide reimbursement for Covered Services under the Customer's Benefit Plan, and authorized by Carrier to access Medical Group's services under this Agreement.

- 1.10 **"Protocols"** are the programs, protocols and administrative procedures adopted by Carrier or a Payer to be followed by Medical Group in providing services and doing business with Carrier and Payers under this Agreement. These Protocols may include, among other things, credentialing and recredentialing processes, utilization management and care management processes, quality improvement, peer review, Customer grievance, concurrent review, or other similar Carrier or Payer programs. The Protocols may change from time to time as discussed in section 5.4 of this Agreement.

Article II.

Representations and Warranties

- 2.1 **Representations and Warranties of Medical Group.** Medical Group, by virtue of its execution and delivery of this Agreement, represents and warrants as follows:

(a) Medical Group is a duly organized and validly existing legal entity in good standing under the laws of its jurisdiction of organization.

(b) Medical Group has all requisite corporate power and authority to conduct its business as presently conducted, and to execute, deliver and perform its obligations under this Agreement. The execution, delivery and performance of this Agreement by Medical Group have been duly and validly authorized by all action necessary under its organizational documents and applicable corporate law.

(c) The execution, delivery and performance of this Agreement by Medical Group do not and will not violate or conflict with (i) the organizational documents of Medical Group, (ii) any material agreement or instrument to which Medical Group is a party or by which Medical Group or any material part of its property is bound, or (iii) applicable law. Medical Group has the unqualified authority to bind, and does bind, itself and Medical Group Professionals to all of the terms and conditions of this Agreement, including any Appendices, Attachments and Exhibits, as applicable.

(d) Medical Group has obtained and holds all registrations, permits, licenses, and other approvals and consents, and has made all filings, that it is required to obtain from or make with all governmental entities under applicable law in order to conduct its business as presently conducted and to enter into and perform its obligations under this Agreement.

(e) Medical Group has been given an opportunity to review the Protocols and Payment Policies and acknowledges that it is bound by the Protocols and that claims under this Agreement will be paid in accordance with the Payment Policies. See the Additional Manuals Appendix for additional information regarding the Protocols and Payment Policies applicable to Customers enrolled in certain Benefit Plans.

(f) Each submission of a claim by Medical Group pursuant to this Agreement shall be deemed to constitute the representation and warranty by it to Carrier that (i) the representations and warranties of it set forth in this section 2.1 and elsewhere in this Agreement are true and correct as of the date the claim is submitted, (ii) it has complied with the requirements of this Agreement with respect to the Covered Services involved and the submission of such claim, (iii) the charge amount set forth on the claim is the Customary Charge and (iv) the claim is a valid claim.

- 2.2 **Representations and Warranties of Carrier.** Carrier, by virtue of its execution and delivery of this Agreement, represents and warrants as follows:

(a) Carrier is a duly organized and validly existing legal entity in good standing under the laws of its jurisdiction of organization.

(b) Carrier has all requisite corporate power and authority to conduct its business as presently conducted, and to execute, deliver and perform its obligations under this Agreement. The execution, delivery and performance of this Agreement by Carrier have been duly and validly authorized by all action necessary under its organizational documents and applicable corporate law.

(c) The execution, delivery and performance of this Agreement by Carrier do not and will not violate or conflict with (i) the organizational documents of Carrier, (ii) any material agreement or instrument to which Carrier is a party or by which Carrier or any material part of its property is bound, or (iii) applicable law.

(d) Carrier has obtained and holds all registrations, permits, licenses, and other approvals and consents, and has made all filings, that it is required to obtain from or make with all governmental entities under applicable law in order to conduct its business as presently conducted and to enter into and perform its obligations under this Agreement.

Article III.

Applicability of this Agreement

- 3.1 Medical Group's Services.** This Agreement applies to Medical Group's practice locations set forth in Appendix 1. In the event Medical Group begins providing services at other locations (either by opening such locations itself, or by acquiring, merging or coming under common ownership and control with an existing provider of services that was not already under contract with Carrier or a company under common ownership to participate in a network of health care providers), such additional locations will become subject to this Agreement 30 days after Carrier receives the notice required under section 5.7(v) of this Agreement.

In the event Medical Group acquires or is acquired by, merges with, or otherwise becomes affiliated with another provider of health care services that is already under contract with Carrier or a company under common ownership to participate in a network of health care providers, this Agreement and the other agreement will each remain in effect and will continue to apply as they did prior to the acquisition, merger or affiliation, unless otherwise agreed to in writing by all parties to such agreements.

Medical Group may transfer all or some of its assets to another entity, if the result of such transfer would be that all or some of the Covered Services subject to this Agreement will be rendered by the other entity rather than by Medical Group, but only if Medical Group requests that Carrier approve the assignment of this Agreement as it relates to those Covered Services and only if the other entity agrees to assume this Agreement. This paragraph does not limit Carrier's right under section 10.4 of this Agreement to elect whether to approve the assignment of this Agreement.

- 3.2 Payers and Benefit Plan types.** Carrier may allow Payers to access Medical Group's services under this Agreement for the Benefit Plan types described in Appendix 2. Appendix 2 may be modified by Carrier upon 30 days written or electronic notice.
- 3.3 Services not covered under a Benefit Plan.** This Agreement does not apply to services not covered under the applicable Benefit Plan. Medical Group may seek and collect payment from a

Customer for such services, provided that the Medical Group first obtains the Customer's written consent.

This section does not authorize Medical Group to bill or collect from Customers for Covered Services for which claims are denied or otherwise not paid. That issue is addressed in sections 7.5 and 7.8 of this Agreement.

- 3.4 Patients who are not Customers.** This Agreement does not apply to services rendered to patients who are not Customers at the time the services were rendered. Section 7.6 of this Agreement addresses circumstances in which claims for services rendered to such persons are inadvertently paid by a Payer.
- 3.5 Health Care.** Medical Group acknowledges that this Agreement and Customer Benefit Plans do not dictate the health care provided by Medical Group or Medical Group Professionals, or govern Medical Group's or Medical Group Professional's determination of what care to provide its patients, even if those patients are Customers. The decision regarding what care is to be provided remains with Medical Group Professionals and with Customers, and not with Carrier or any Payer.
- 3.6 Communication with Customers.** Nothing in this Agreement is intended to limit Medical Group's or Medical Group Professional's right or ability to communicate fully with a Customer regarding the Customer's health condition and treatment options. Medical Group and Medical Group Professionals are free to discuss all treatment options without regard to whether or not a given option is a Covered Service. Medical Group and Medical Group Professionals are free to discuss with a Customer any financial incentives Medical Group may have under this Agreement, including describing at a general level the payment methodologies contained in this Agreement.

Article IV.

Participation of Medical Group Professionals in Carrier's Network

- 4.1 Medical Group Professionals as Participating Providers.** Except as described under section 4.2, all Medical Group Professionals will participate in Carrier's network. Medical Group has the authority to bind, and will bind, all new Medical Group Professionals to the obligations of this Agreement.
- 4.2 Medical Group Professionals who are not Participating Providers.** The following Medical Group Professionals are not participating providers in Carrier's network:
- i) A Medical Group Physician (or a Medical Group Non-Physician Provider, in the event such provider is of a provider type that Carrier credentials) who has been denied participation in Carrier's credentialing program, whose credentialing application has not been submitted, or whose credentialing application remains pending; or
 - ii) A Medical Group Professional who has been terminated from participation in Carrier's network pursuant to section 4.5 of this Agreement.
- 4.3 Credentialing.** Medical Group and Medical Group Physicians will participate in and cooperate with Carrier's credentialing program (subject to the delegated credentialing agreement between the parties, so long as that agreement remains in effect). Medical Group Non-Physician Providers

will participate in and cooperate with Carrier's credentialing program to the extent such Medical Group Non-Physician Providers are subject to credentialing by Carrier.

- 4.4 New Medical Group Professionals.** Medical Group will notify Carrier at least 30 days before a physician becomes a Medical Group Physician. In the event that the Medical Group's agreement with the new Medical Group Physician provides for a starting date that would make it impossible for Medical Group to provide 30 days advance notice to Carrier, then Medical Group will give notice to Carrier within five business days after reaching agreement with the new Medical Group Physician. In either case, the new Medical Group Physician will submit and complete a credentialing application to Carrier within 30 days of the new Medical Group Physician's agreement to join Medical Group, unless the new Medical Group Physician already has been credentialed by Carrier and is already a participant in Carrier's network.

The requirements of this section 4.4 also apply to new Medical Group Non-Physician Providers who are subject to credentialing by Carrier.

- 4.5 Termination of a Medical Group Professional from Carrier's Network.** Carrier may terminate a Medical Group Professional's participation in Carrier's network, without terminating this Agreement, immediately upon becoming aware of any of the following:

- i) material breach of this Agreement that is not cured by Medical Group Professional within 30 days after Carrier provided notice to Medical Group of the breach;
- ii) the suspension, revocation, condition, limitation, qualification or other material restriction on a Medical Group Professional's licenses, certifications and permits by any government agency under which the Medical Group Professional is authorized to provide health care services;
- iii) the suspension, revocation, condition, limitation, qualification or other material restriction of a Medical Group Physician's staff privileges at any licensed hospital, nursing home or other facility at which the Medical Group Physician has staff privileges during the term of this Agreement;
- iv) an indictment, arrest or conviction for a felony, or for any criminal charge related to the practice of Medical Group Professional's profession;
- v) a sanction imposed by any governmental agency or authority, including Medicare or Medicaid; or
- vi) pursuant to Carrier's Credentialing Plan.

Carrier will notify Medical Group of the Medical Group Professional's termination according to the notice provision set forth in section 10.8 of this Agreement.

- 4.6 Covered Services by Medical Group Professionals who are not Participating Providers.** Medical Group will staff its service locations so that Covered Services can appropriately be rendered to Customers by Medical Group Professionals who participate in Carrier's network. A Medical Group Professional who does not participate in Carrier's network, pursuant to section 4.2 of this Agreement, will not render Covered Services to a Customer.

In the event Covered Services are rendered by a Medical Group Professional who does not participate in Carrier's network, Medical Group and the Medical Group Professional will not submit a claim or other request for payment to Carrier or Payer, and will not seek or accept payment from the Customer.

Article V.
Duties of Medical Group

- 5.1 Provide Covered Services.** Medical Group will provide Covered Services to Customers.
- 5.2 Nondiscrimination.** Medical Group will not discriminate against any patient, with regard to quality of service or accessibility of services, on the basis that the patient is a Customer. Medical Group will not require a Customer to pay a "membership fee" or other fee in order to access Medical Group for Covered Services (except for co-payments, coinsurance and/or deductibles provided for under Customer's Benefit Plan) and will not discriminate against any Customer based on the failure to pay such a fee.
- 5.3 Accessibility.** Medical Group will provide or arrange for the provision of advice and assistance to Customers in emergency situations 24 hours a day, seven days a week.
- 5.4 Cooperation with Protocols.** Medical Group will cooperate with and comply with Carrier's and Payers' Protocols subject to any applicable Regulatory Appendix. The Protocols include but are not limited to all of the following:
1. Medical Group will use reasonable commercial efforts to direct Customers only to other providers that participate in Carrier's network, except as otherwise authorized by Carrier or Payer.
 2. If the Customer's Benefit Plan requires the Customer to receive certain Covered Services from or upon referral by a primary care physician, all referral physicians must adhere to the following additional protocols when those Covered Services are provided:
 - a. Notify Customer's primary care physician of referrals to other participating or non-participating providers.
 - b. Covered Services must be provided pursuant to the terms and limitations of the referral notification issued by or on behalf of the Customer's primary care physician.
 - c. If the Medical Group Physician providing the Covered Services is a referral physician, the Medical Group Physician must also notify the Customer's primary care physician of all admissions in accordance with the required time frames.
 3. Medical Group will provide notification for certain Covered Services, accept and return telephone calls from Carrier staff, and respond to Carrier requests for clinical information, as required by Carrier or Payer as described in the Protocols.

The Protocols will be made available to Medical Group online or upon request. Some or all Protocols also may be disseminated in the form of an administrative manual or guide or in other communications.

Carrier may change the Protocols from time to time. Carrier will use reasonable commercial efforts to inform Medical Group at least 30 days in advance of any material changes to the Protocols. Carrier may implement changes in the Protocols without Medical Group's consent if such change is applicable to all or substantially all of the medical groups in Carrier's network located in the same state as Medical Group and that practice the same specialty as Medical

Group. Otherwise, changes to the Protocols proposed by Carrier to be applicable to Medical Group are subject to the terms of section 10.2 of this Agreement that are applicable to amendments.

- 5.5 Licensure.** Medical Group and Medical Group Professionals will maintain, without material restriction, such licensure, registration, and permits as are necessary to enable Medical Group and Medical Group Professionals to lawfully perform this Agreement.
- 5.6 Liability Insurance.** Medical Group will assure that Medical Group and all Medical Group Professionals are covered by liability insurance. Except to the extent coverage is a state mandated placement, the liability coverage must be placed with responsible, financially sound insurance carriers authorized or approved to write coverage in the state in which the Covered Services are provided. The liability insurance shall be, at a minimum, of the types and in the amounts set forth below. Medical malpractice insurance shall be either occurrence or claims made with an extended period reporting option. Prior to the Effective Date of this Agreement and within 10 days of each policy renewal thereafter, Medical Group shall submit to Carrier in writing evidence of insurance coverage.

<u>TYPE OF INSURANCE</u>	<u>MINIMUM LIMITS</u>
Medical malpractice and/or professional liability insurance	Three Million Dollars (\$3,000,000) per occurrence and Five Million Dollars (\$5,000,000) aggregate, if Medical Group insures all Medical Group Professionals in a single policy. This insurance requirement will also be satisfied if the Medical Group insures each Medical Group Professional separately, and the coverage for each Medical Group Professional is at least One Million Dollars (\$1,000,000) per occurrence and Three Million Dollars (\$3,000,000) aggregate.
Commercial general and/or umbrella liability insurance	One Million Dollars (\$1,000,000) per occurrence and aggregate.

In lieu of purchasing the insurance coverage required in this section, Medical Group may, with the prior written approval of Carrier, self-insure its medical malpractice and/or professional liability, as well as its commercial general liability. Medical Group shall maintain a separate reserve for its self-insurance. If Medical Group will use the self-insurance option described in this paragraph, Medical Group will provide to Carrier, prior to the Effective Date, a statement verified by an independent auditor or actuary that its reserve funding levels and process of funding appears adequate to meet the requirements of this section and fairly represents the financial condition of the fund. Medical Group will provide a similar statement during the term of this Agreement upon Carrier's request, which will be made no more frequently than annually. Medical Group will assure that its self-insurance fund will comply with applicable laws and regulations.

- 5.7 Notice.** Medical Group will give notice to Carrier within 10 days after any event that causes Medical Group to be out of compliance with section 5.5 or 5.6 of this Agreement, or of any change in Medical Group's name, ownership, control, or Taxpayer Identification Number. This section does not apply to changes of ownership or control that result in Medical Group being owned or controlled by an entity with which it was already affiliated prior to the change. In

addition, Medical Group will give written notice to Carrier within 10 days after it learns of any of the following:

- i) any suspension, revocation, condition, limitation, qualification or other material restriction on a Medical Group Professional's licenses, certifications and permits by any government agency under which a Medical Group Professional is authorized to provide health care services;
- ii) any suspension, revocation, condition, limitation, qualification or other material restriction of a Medical Group Physician's staff privileges at any licensed hospital, nursing home or other facility at which a Medical Group Physician has staff privileges during the term of this Agreement;
- iii) indictment, arrest or conviction of a Medical Group Professional for a felony, or for any criminal charge related to the practice of the Medical Group Professional's profession;
- iv) the departure of any Medical Group Professional from Medical Group; or
- v) any changes to the information contained in Appendix 1.

5.8 Customer consent to release of Medical Record Information. Medical Group will obtain any Customer consent required in order to authorize Medical Group to provide access to requested information or records as contemplated in section 5.9 of this Agreement, including copies of the Medical Group's medical records relating to the care provided to Customer.

5.9 Maintenance of and Access to Records. Medical Group will maintain adequate medical, financial and administrative records related to Covered Services rendered by Medical Group under this Agreement, including claims records, for at least 6 years following the end of the calendar year during which the Covered Services are provided, unless a longer retention period is required by applicable law.

Medical Group will provide access to these records as follows:

- i) to Carrier or its designees, in connection with Carrier's utilization management/care management, quality assurance and improvement and for claims payment and other administrative obligations, including reviewing Medical Group's compliance with the terms and provisions of this Agreement and appropriate billing practice. Medical Group will provide access during ordinary business hours within fourteen days after a request is made, except in cases of a Carrier audit involving a fraud investigation or the health and safety of a Customer (in which case, access shall be given within 48 hours after the request) or of an expedited Customer appeal or grievance (in which case, access will be given so as to enable Carrier to reasonably meet the timelines for determining the appeal or grievance); and
- ii) to agencies of the government, in accordance with applicable law, to the extent such access is necessary to comply with regulatory requirements applicable to Medical Group, Carrier, or Payers.

Medical Group will cooperate with Carrier on a timely basis in connection with any such audit including, among other things, in the scheduling of and participation in an audit exit interview within 30 days of Carrier's request.

Upon invoice from Medical Group, Carrier will pay for copies of records requested by Carrier in cases where Carrier requests the records more than once. Payment will be made at a rate of 25 per page, not to exceed a total of 25 per record, unless a different rate is specified under state law.

- 5.10 Access to Data.** Medical Group represents that in conducting its operations, it collects and reviews certain quality data relating to care rendered by Medical Group that is reported in a manner which has been validated by a third party as having a clear, evidence-based link to quality or safety (e.g., AHRQ standards) or which has been created by employer coalitions as proxies for quality (e.g., Leapfrog standards).

Carrier recognizes that Medical Group has the sole discretion to select the metrics which it will track from time to time and that Medical Group's primary goal in so tracking is to advance the quality of patient care. On a quarterly basis, Medical Group will share these metrics with Carrier as tracked against a database of all commercial patients (including patients who are not Carrier customers). Carrier may publish this data to entities to which Carrier renders services or seeks to render services, and to Customers.

- 5.11 Compliance with law.** Medical Group will comply with applicable regulatory requirements, including but not limited to those relating to confidentiality of Customer medical information.
- 5.12 Electronic connectivity.** When made available by Carrier, Medical Group will do business with Carrier electronically. Medical Group will use www.unitedhealthcareonline.com to check eligibility status, claims status, and submit requests for claims adjustments for Customers enrolled in products supported by www.unitedhealthcareonline.com. Medical Group agrees to use www.unitedhealthcareonline.com for additional functionalities (for instance, notification of admission) after Carrier informs Medical Group that such functionalities have become available for the applicable Customer.
- 5.13 Employees and subcontractors.** Medical Group will assure that its employees, affiliates and any individuals or entities subcontracted by Medical Group to render services in connection with this Agreement adhere to the requirements of this Agreement. The use of employees, affiliates or subcontractors to render services in connection with this Agreement will not limit Medical Group's obligations and accountability under this Agreement with regard to such services.

For laboratory services, Medical Group will only be reimbursed for services that Medical Group is certified through the Clinical Laboratory Improvement Amendments (CLIA) to perform, and Medical Group must not bill Customers for any laboratory services for which Medical Group lacks CLIA certification.

Article VI.

Duties of Carrier and Payers

- 6.1 Payment of Claims.** As described in further detail in Article VII of this Agreement, Payers will pay Medical Group for rendering Covered Services to Customers.
- 6.2 Liability Insurance.** Carrier will procure and maintain professional and general liability insurance and other insurance, as Carrier reasonably determines may be necessary, to protect Carrier and Carrier's employees against claims, liabilities, damages or judgments that arise out of services provided by Carrier or Carrier's employees under this Agreement.
- 6.3 Licensure.** Carrier will maintain, without material restriction, such licensure, registration, and permits as are necessary to enable Carrier to lawfully perform this Agreement.
- 6.4 Notice.** Carrier will give written notice to Medical Group within 10 days after any event that causes Carrier to be out of compliance with section 6.2 or 6.3 of this Agreement, or of any change

in Carrier's name, ownership, control, or Taxpayer Identification Number. This section does not apply to changes of ownership or control that result in Carrier being owned or controlled by an entity with which it was already affiliated prior to the change.

- 6.5 Compliance with law.** Carrier will comply with applicable regulatory requirements, including but not limited to those relating to confidentiality of Customer medical information and those relating to prompt payment of claims, to the extent those requirements are applicable.
- 6.6 Electronic connectivity.** Carrier will communicate with Medical Group electronically by providing eligibility status, claims status, and accepting requests for claim adjustments, for those products supported by www.unitedhealthcareonline.com. Carrier will communicate enhancements in www.unitedhealthcareonline.com functionality as they become available, as described in Section 5.12, and will make information available as to which products are supported by www.unitedhealthcareonline.com.
- 6.7 Employees and subcontractors.** Carrier will assure that its employees, affiliates and any individuals or entities subcontracted by Carrier to render services in connection with this Agreement adhere to the requirements of this Agreement. The use of employees, affiliates or subcontractors to render services in connection with this Agreement will not limit Carrier's obligations and accountability under this Agreement with regard to such services.

Article VII.

Submission, Processing, and Payment of Claims

- 7.1 Form and content of claims.** Medical Group must submit claims for Covered Services in a manner and format prescribed by Carrier, as further described in the Protocols. Unless otherwise directed by Carrier, Medical Group shall submit claims using current CMS 1500 or UB04 or successor forms for paper claims and HIPAA standard professional or institutional claim formats for electronic claims, as applicable, with applicable coding including, but not limited to, ICD, CPT, Revenue and HCPCS coding.
- Medical Group will submit claims only for services performed by Medical Group or Medical Group staff. Pass through billing is not payable under this Agreement.
- 7.2 Electronic filing of claims.** Within six months after the Effective Date of this Agreement, Medical Group will use electronic submission for all of its claims under this Agreement that Carrier is able to accept electronically.
- 7.3 Time to file claims.** All information necessary to process a claim must be received by Carrier no more than 90 days from the date that Covered Services are rendered. In the event Carrier requests additional information in order to process the claim, Medical Group will provide such additional information within 90 days of Carrier's request. If Payer is not the primary payer, and Medical Group is pursuing payment from the primary payer, the 90 day filing limit will begin on the date Medical Group receives the claim response from the primary payer.
- 7.4 Payment of claims.** Payer will pay claims for Covered Services according to the lesser of Medical Group's Customary Charge or the applicable fee schedule (as further described in Appendix 3 to this Agreement), and in accordance with Payment Policies.

Claims for Covered Services subject to coordination of benefits will be paid in accordance with the Customer's Benefit Plan and applicable state and federal law.

The obligation for payment under this Agreement is solely that of Payer, and not that of Carrier unless Carrier is the Payer.

Ordinarily, fee amounts listed in Appendix 3 are based upon primary fee sources. Carrier reserves the right to use gap-fill fee sources where primary fee sources are not available.

Carrier routinely updates its fee schedule in response to additions, deletions, and changes to CPT codes by the American Medical Association, price changes for immunizations and injectable medications, and in response to similar changes (additions and revisions) to other service coding and reporting conventions that are widely used in the health care industry, such as those maintained by the Centers for Medicare and Medicaid Services (for example HCPCS, etc.). Ordinarily, Carrier's fee schedule is updated using similar methodologies for similar services. Carrier will not generally attempt to communicate routine maintenance of this nature and will generally implement updates within 90 days from the date of publication.

Carrier will give Medical Group 90 days written or electronic notice of non-routine fee schedule changes which will substantially alter the overall methodology or reimbursement level of the fee schedule. In the event such changes will reduce Medical Group's overall reimbursement under this Agreement, Medical Group may terminate this Agreement by giving 60 days written notice to Carrier, provided that the notice is given by Medical Group within 60 days after the notice of the fee schedule change.

Carrier will make its Payment Policies available to Medical Group online or upon request. Carrier may change its Payment Policies from time to time.

7.5 Denial of Claims for Not Following Protocols, Not Filing Timely, or Lack of Medical Necessity. Payment may be denied in whole or in part if Medical Group does not comply with a Protocol or does not file a timely claim under section 7.3 of this Agreement. Payment may also be denied for services provided that are determined by Carrier to be medically unnecessary, and Medical Group may not bill the Customer for such services unless the Customer has, with knowledge of Carrier's determination of a lack of medical necessity, agreed in writing to be responsible for payment of those charges.

In the event that payment of a claim is denied for lack of notification or for untimely filing, the denial will be reversed if Medical Group appeals within 12 months after the date of denial and can show all of the following:

- i) that, at the time the Protocols required notification or at the time the claim was due, Medical Group did not know and was unable to reasonably determine that the patient was a Customer,
- ii) that Medical Group took reasonable steps to learn that the patient was a Customer, and
- iii) that Medical Group promptly provided notification, or filed the claim, after learning that the patient was a Customer.

7.6 Retroactive Correction of Information Regarding Whether Patient Is a Customer. Prior to rendering services, Medical Group will ask the patient to present his or her Customer

identification card. In addition, Medical Group may contact Carrier to obtain the most current information on the patient as a Customer.

However, Medical Group acknowledges that such information provided by Carrier is subject to change retroactively, under the following circumstances: (1) if Carrier has not yet received information that an individual is no longer a Customer; (2) if the individual's Benefit Plan is terminated retroactively for any reason including, but not limited to, non-payment of premium; (3) as a result of the Customer's final decision regarding continuation of coverage pursuant to state and federal laws; or (4) if eligibility information Carrier receives is later proven to be false.

If Medical Group provides health care services to an individual, and it is determined that the individual was not a Customer at the time the health care services were provided, those services shall not be eligible for payment under this Agreement and any claims payments made with regard to such services may be recovered as overpayments under the process described in section 7.10 of this Agreement. Medical Group may then directly bill the individual, or other responsible party, for such services.

7.7 Payment under this Agreement is Payment in Full. Payment as provided under section 7.4 of this Agreement, together with any co-payment, deductible or coinsurance for which the Customer is responsible under the Benefit Plan, is payment in full for a Covered Service. Medical Group will not seek to recover, and will not accept any payment from Customer, Carrier, Payer or anyone acting in their behalf, in excess of payment in full as provided in this section 7.7, regardless of whether such amount is less than Medical Group's billed charge or Customary Charge.

7.8 Customer "Hold Harmless." Medical Group will not bill or collect payment from the Customer, or seek to impose a lien, for the difference between the amount paid under this Agreement and Medical Group's billed charge or Customary Charge, or for any amounts denied or not paid under this Agreement due to:

- i) Medical Group's failure to comply with the Protocols,
- ii) Medical Group's failure to file a timely claim,
- iii) Payer's Payment Policies,
- iv) inaccurate or incorrect claim processing,
- v) insolvency or other failure by Payer to maintain its obligation to fund claims payments, if Payer is Carrier, or is an entity required by applicable law to assure that its Customers not be billed in such circumstances, or
- vi) a denial based on medical necessity or prior authorization, except as permitted under section 7.5.

This obligation to refrain from billing Customers applies even in those cases in which Medical Group believes that Carrier or Payer has made an incorrect determination. In such cases, Medical Group may pursue remedies under this Agreement against Carrier or Payer, as applicable, but must still hold the Customer harmless.

In the event of a default by a Payer other than those Payers covered by the above clause v), Medical Group may seek payment directly from the Payer or from Customers covered by that Payer. However, Medical Group may do so only if it first inquires in writing to Carrier as to whether the Payer has defaulted and, in the event that Carrier confirms that Payer has defaulted (which confirmation will not be unreasonably withheld), Medical Group then gives Carrier 15 days prior written notice of Medical Group's intent to seek payment from Payer or Customers. For purposes of this paragraph, a default is a systematic failure by a Payer to fund claims

payments related to Customers covered through that Payer; a default does not occur in the case of a dispute as to whether certain claims should be paid or the amounts that should be paid for certain claims.

This section 7.8 and section 7.7 will survive the termination of this Agreement, with regard to Covered Services rendered prior to when the termination takes effect.

- 7.9 Consequences for Failure to Adhere to Customer Protection Requirements.** If Medical Group collects payment from, brings a collection action against, or asserts a lien against a Customer for Covered Services rendered (other than for the applicable co-payment, deductible or coinsurance), contrary to section 7.7 or 7.8 of this Agreement, Medical Group shall be in breach of this Agreement. This section 7.9 will apply regardless of whether Customer or anyone purporting to act on Customer's behalf has executed a waiver or other document of any kind purporting to allow Medical Group to collect such payment from Customer.

In the event of such a breach, Payer may deduct, from any amounts otherwise due Medical Group, the amount wrongfully collected from Customers, and may also deduct an amount equal to any costs or expenses incurred by the Customer, Carrier or Payer in defending the Customer from such action and otherwise enforcing sections 7.7 through 7.9 of this Agreement. Any amounts deducted by Payer in accordance with this provision shall be used to reimburse the Customer and to satisfy any costs incurred. The remedy contained in this paragraph does not preclude Carrier from invoking any other remedy for breach that may be available under this Agreement.

- 7.10 Correction of Overpayments or Underpayments of Claims.** In the event that either Party believes that a claim has not been paid correctly, or that funds were paid beyond or outside of what is provided for under this Agreement, either party may seek correction of the payment by giving the other party notice, within 12 months after the payment was initially made, that it believes the payment was made incorrectly.

The 12 month limit will not apply to recovery of overpayments in either of the following circumstances:

- (i) the overpayment resulted from fraud or abuse by or on behalf of Medical Group, or
- (ii) Carrier's ability to discover the overpayment during the 12 month period was hindered by Medical Group's failure to provide full and timely cooperation with an audit by Carrier.

Medical Group will repay overpayments within 30 days of notice of the overpayment. Medical Group will promptly report any credit balance that it maintains with regard to any claim overpayment under this Agreement, and will return such overpayment to Carrier within 30 days after posting it as a credit balance.

Medical Group agrees that recovery of overpayments may be accomplished by offsets against future payments.

- 7.11 Claims Payment Issues Arising from Departure of Medical Group Professionals from Medical Group.** In the event a Medical Group Professional departs from Medical Group and uncertainty arises as to whether Medical Group or some other entity is entitled to receive payment for certain services rendered by such former Medical Group Professional, the parties will cooperate with each other in good faith in an attempt to resolve the situation appropriately.

In the event that Medical Group's failure to give timely notice under section 5.7 (iv) of this Agreement resulted in claims payments being made incorrectly to Medical Group, Medical Group shall promptly call the situation to Carrier's attention and return such payments to Carrier. In the event Medical Group fails to do so, Carrier may hold Medical Group liable for any attorneys fees, costs, or administrative expenses incurred by Carrier as a result.

In the event that both Medical Group and some other entity assert a right to payment for the same service rendered by the former Medical Group Professional, Carrier may refrain from paying either entity until the payment obligation is clarified. Provided that Carrier acts in good faith, Medical Group will waive any right to receive interest or penalties under any applicable law relating to the prompt payment of claims.

Article VIII. **Dispute Resolution**

The parties will work together in good faith to resolve any and all disputes between them (hereinafter referred to as "Disputes") including but not limited to all questions of arbitrability, the existence, validity, scope or termination of the Agreement or any term thereof.

If the parties are unable to resolve any such Dispute within 60 days following the date one party sent written notice of the Dispute to the other party, and if either party wishes to pursue the Dispute, it shall thereafter be submitted to binding arbitration in accordance with the Commercial Dispute Procedures of the American Arbitration Association, as they may be amended from time to time (see <http://www.adr.org>). Unless otherwise agreed to in writing by the parties, the party wishing to pursue the Dispute must initiate the arbitration within one year after the date on which notice of the Dispute was given or shall be deemed to have waived its right to pursue the dispute in any forum.

Any arbitration proceeding under this Agreement shall be conducted in King County, WA. The arbitrator(s) may construe or interpret but shall not vary or ignore the terms of this Agreement and shall be bound by controlling law. The arbitrator(s) shall have no authority to award punitive, exemplary, indirect or special damages, except in connection with a statutory claim that explicitly provides for such relief.

The parties expressly intend that any dispute relating to the business relationship between them be resolved on an individual basis so that no other dispute with any third party(ies) may be consolidated or joined with the Dispute. The parties agree that any arbitration ruling by an arbitrator allowing class action arbitration or requiring consolidated arbitration involving any third party(ies) would be contrary to their intent and would require immediate judicial review of such ruling.

If the Dispute pertains to a matter which is generally administered by certain Carrier procedures, such as a credentialing or quality improvement plan, the policies and procedures set forth in that plan must be fully exhausted by Medical Group before Medical Group may invoke any right to arbitration under this Article VIII.

The decision of the arbitrator(s) on the points in dispute will be binding, and judgment on the award may be entered in any court having jurisdiction thereof. The parties acknowledge that because this Agreement affects interstate commerce the Federal Arbitration Act applies.

In the event that any portion of this Article or any part of this Agreement is deemed to be unlawful,

invalid or unenforceable, such unlawfulness, invalidity or unenforceability shall not serve to invalidate any other part of this Article or Agreement. In the event any court determines that this arbitration procedure is not binding or otherwise allows litigation involving a Dispute to proceed, the parties hereby waive any and all right to trial by jury in, or with respect to, such litigation. Such litigation would instead proceed with the judge as the finder of fact.

In the event a party wishes to terminate this Agreement based on an assertion of uncured material breach, and the other party disputes whether grounds for such a termination exist, the matter will be resolved through arbitration under this Article VIII. While such arbitration remains pending, the termination for breach will not take effect.

This Article VIII governs any dispute between the parties arising before or after execution of this Agreement and shall survive any termination of the Agreement.

Article IX. **Term and Termination**

9.1 Term. This Agreement shall take effect on the Effective Date. This Agreement shall have an initial term of three years and renew automatically for renewal terms of one year, until terminated pursuant to section 9.2.

9.2 Termination. This Agreement may be terminated under any of the following circumstances:

- i) by mutual written agreement of the parties;
- ii) by either party, upon at least 90 days prior written notice effective at the end of the initial term or effective at the end of any renewal term;
- iii) by either party upon 60 days written notice in the event of a material breach of this Agreement by the other party, except that such a termination will not take effect if the breach is cured within 60 days after notice of the termination; moreover, such termination may be deferred as further described in Article VIII of this Agreement;
- iv) by either party upon 10 days written notice in the event the other party loses licensure or other governmental authorization necessary to perform this Agreement, or fails to have insurance as required under section 5.6 or section 6.2 of this Agreement; or
- v) by Medical Group, as described in section 7.4 of this Agreement in the event of a non-routine fee schedule change.

9.3 Ongoing Services to Certain Customers After Termination Takes Effect. In the event a Customer is receiving any of the Covered Services listed below, as of the date the termination takes effect, Medical Group will continue to render those Covered Services to that Customer and this Agreement will continue to apply to those Covered Services, after the termination takes effect, for the length of time indicated below:

Inpatient Covered Services	30 days or until discharge, whichever comes first
Pregnancy, Third Trimester – Low Risk	Through postpartum follow up visit
Pregnancy, First, Second or Third Trimester – Moderate Risk and High Risk	Through postpartum follow up visit
Non-Surgical Cancer Treatment	30 days or a complete cycle of radiation or chemotherapy, whichever is greater
End Stage Kidney Disease and Dialysis	30 days
Symptomatic AIDS undergoing active	30 days

treatment	
Circumstances where Payer is required by applicable law to provide transition coverage of services rendered by Medical Group after Medical Group leaves the provider network accessed by Payer.	As applicable

Article X.
Miscellaneous Provisions

- 10.1 Entire Agreement.** This Agreement is the entire agreement between the parties with regard to the subject matter herein, and supersedes any prior written or unwritten agreements between the parties or their affiliates with regard to the same subject matter.
- 10.2 Amendment.** Carrier can amend this Agreement or any of the appendices on 90 days written or electronic notice by sending Medical Group a copy of the amendment. Medical Group's signature is not required to make the amendment effective. However, if the amendment is not required by law or regulation and would impose a material adverse impact on Medical Group, then Medical Group may terminate this Agreement on 60 days written notice to Carrier by sending a termination notice within 30 days after receipt of the amendment.
- 10.3 Nonwaiver.** The waiver by either party of any breach of any provision of this Agreement shall not operate as a waiver of any subsequent breach of the same or any other provision.
- 10.4 Assignment.** This Agreement may not be assigned by either party without the written consent of the other party, except that this Agreement may be assigned by Carrier to a company under common ownership with Carrier.
- 10.5 Relationship of the Parties.** The sole relationship between the parties to this Agreement is that of independent contractors. This Agreement does not create a joint venture, partnership, agency, employment or other relationship between the parties.
- 10.6 No Third-Party Beneficiaries.** Carrier and Medical Group are the only entities with rights and remedies under the Agreement.
- 10.7 Delegation.** Carrier may delegate (but not assign) certain of its administrative duties under this Agreement to one or more other entities. No such delegation will relieve Carrier of its obligations under this Agreement.
- 10.8 Notice.** Any notice required to be given under this Agreement shall be in writing, except in cases in which this Agreement specifically permits electronic notice, or as otherwise permitted or required in the Protocols. All written or electronic notices shall be deemed to have been given when delivered in person, by electronic communication, by facsimile or, if delivered by first-class United States mail, on the date mailed, proper postage prepaid and properly addressed to the appropriate party at the address set forth on the signature portion of this Agreement or to another more recent address of which the sending party has received written notice. Notwithstanding the previous sentence, all notices of termination of this Agreement by either party must be sent by certified mail, return receipt requested.

Each party shall provide the other with proper addresses, facsimile numbers and electronic mail
UHC/MGA.09.09.WA Parties are bound to confidentiality provisions stated under Section 10.9 of this Agreement

addresses of all designees that should receive certain notices or communication instead of that party.

10.9 Confidentiality. Neither party will disclose to a Customer, other health care providers, or other third parties any of the following information (except as required by an agency of the government):

- i) any proprietary business information, not available to the general public, obtained by the party from the other party;
- ii) the specific reimbursement amounts provided for under this Agreement, except for purposes of administration of benefits.

At least 48 hours before either party issues a press release, advertisement, or other media statement about the business relationship between the parties, that party will give the other party a copy of the material the party intends to issue.

10.10 Governing Law. This Agreement will be governed by and construed in accordance with the laws of the state in which Medical Group renders Covered Services, and any other applicable law.

10.11 Regulatory Appendices. One or more regulatory appendix may be attached to this Agreement, setting forth additional provisions included in this Agreement in order to satisfy regulatory requirements under applicable law. These regulatory appendices, and any attachments to them, are expressly incorporated into this Agreement and are binding on the parties to this Agreement. In the event of any inconsistent or contrary language between a regulatory appendix and any other part of this Agreement, including but not limited to appendices, amendments and exhibits, the provisions of the regulatory appendix will control, to the extent it is applicable.

10.12 Severability. Any provision of this Agreement that is unlawful, invalid or unenforceable in any situation in any jurisdiction shall not affect the validity or enforceability of the remaining provisions of this Agreement or the lawfulness, validity or enforceability of the offending provision in any other situation or jurisdiction.

10.13 Survival. Sections 5.9, 7.7, 7.8, Article VIII and sections 9.3 and 10.9 (except for the last paragraph) of this Agreement will survive the termination of this Agreement.

Attachments

- ☒ Appendix 1: Medical Group Practice Locations
- ☒ Appendix 2: Benefit Plan Descriptions
- ☒ Appendix 3: Fee Schedule Sample
- ☒ Additional Manuals Appendix
- ☒ State Regulatory Requirements Appendix (Appendices) (list all states as applicable)
 - ☐ WA
 - ☐
 - ☐
 - ☐
- ☒ Medicare Advantage Regulatory Requirements Appendix
- ☒ Medicaid Regulatory Requirements Appendix
- ☐ Other

Appendix 1 Medical Group Practice Locations

Medical Group attests that this Appendix identifies all services and locations covered under this Agreement.

IMPORTANT NOTE: Medical Group acknowledges its obligation under Section 5.7 to promptly report any change in Medical Group's name or Taxpayer Identification Number. Failure to do so may result in denial of claims or incorrect payment.

BILLING ADDRESS

Practice Name RADIANTCARE RADIATION ONCOLOGY LLC
 Street Address 4525 3RD AVE SE STE 100
 City LACEY State WA Zip 98503-1010
 Tax ID Number (TIN) 912018691

PRACTICE LOCATIONS (complete one for each service location)		
Clinic Name	Clinic Name	Clinic Name
RADIANTCARE LACEY	RADIANTCARE ABERDEEN	RADIANTCARE CENTRALIA
Street Address	Street Address	Street Address
4525 3RD AVE SE STE 100	1200 BASICH BLVD	2015 COOKS HILL RD
City	City	City
LACEY	ABERDEEN	CENTRALIA
State and Zip Code	State and Zip Code	State and Zip Code
WA 98503-1010	WA 98520-1035	WA 98531-9074
Phone Number	Phone Number	Phone Number
360-412-8960	360-537-8801	360-736-8960
TIN (If different from above)	TIN (If different from above)	TIN (If different from above)

PRACTICE LOCATIONS (complete one for each service location)		
Clinic Name	Clinic Name	Clinic Name
Street Address	Street Address	Street Address
City	City	City
State and Zip Code	State and Zip Code	State and Zip Code
Phone Number	Phone Number	Phone Number
TIN (If different from above)	TIN (If different from above)	TIN (If different from above)

Appendix 2 Benefit Plan Descriptions

1. Carrier may allow Payers to access Medical Group's services under this Agreement for the Benefit Plan types described below:

- Benefit Plans where Customers are offered a network of participating providers and must select a primary physician. Such Benefit Plans may or may not include an out-of-network benefit.
- Benefit Plans where Customers are offered a network of participating providers but are not required to select a primary physician. Such Benefit Plans may or may not include an out-of-network benefit.
- Benefit Plans where Customers are not offered a network of participating providers from which they may receive Covered Services.
- Evercare/SecureHorizons Medicare Benefit Plans.
- Washington Healthy Options Benefit Plans.
- Washington Medicaid Benefit Plans.
- Washington CHIP Benefit Plans.

2. Medical Group will not participate in the network of physicians and other health care professionals and providers established by Carrier for the Benefit Plan types described below:

- Capitation arrangements other than for Medicare Benefit Plans. (See below for definition of capitation arrangements.)
- AmeriChoice Medicare Benefit Plans.
- Workers' compensation Benefit Plans.
- Washington Other Governmental Benefit Plans.
- TRICARE Benefit Plans.
- Medicare Advantage Private Fee-For-Service Benefit Plans and Medicare Advantage Medical Savings Account Benefit Plans.
- Benefit Plans for Medicare Select.

- Medicaid and CHIP Benefit Plans for states other than Washington.

This Agreement does not supersede any existing agreements between the parties or their affiliates that are not identified above in section 1 of this Appendix. Additionally, this Agreement does not prevent the parties or their affiliates from entering into other agreements in the future.

3. Definitions:

Note- Certain Benefit Plans defined below are defined with reference to specific identifiers on the Customer identification card. Those Benefit Plans may adopt a different identification card identifier in the future, and this Appendix will continue to apply to those Benefit Plans; if that happens, Carrier will provide Medical Group with information regarding the new Customer identification cards.

A. CAPITATION ARRANGEMENTS:

“Capitation arrangements” are when all of the following apply:

- (i) Medical Group (directly or through an IPA or other provider organization in which Medical Group is a participant) is part of a network for Carrier; and
- (ii) As part of that network, Medical Group arranges directly with Carrier, or an IPA or another medical group or an other provider organization, for certain designated services to be provided to members who are assigned to Medical Group or to the IPA or the other medical group or other provider organization (as the case may be) and who are covered by Carrier’s Benefit Plans, and under which either:
 - (a) Medical Group is capitated or otherwise has financial responsibility; or
 - (b) Medical Group is paid on a fee-for-service basis directly by the IPA or by the other medical group or other provider organization which has financial responsibility for the service, at a rate agreed upon by Medical Group with that IPA or other medical group or other provider organization; and
- (iii) Medical Group provides those designated services to one of those assigned members.

In such cases, the obligation for payment will be primarily that of the medical group, IPA or other provider organization that has financial responsibility for the service and not that of Carrier.

It is not a capitation arrangement when:

- (1) Another medical group or an IPA or other provider organization is not affiliated with Medical Group, and is capitated by Carrier for designated Covered Services rendered to assigned Customers covered by a Benefit Plan issued by Carrier; and
- (2) Medical Group provides those designated Covered Services to one of those assigned Customers, without having a contract or other arrangement with the other medical group or the IPA or other provider organization for the terms under which those designated Covered Services are provided.

In such cases, this Agreement will apply and the medical group or IPA or other provider organization that has responsibility for the Covered Service will be considered the Payer.

MEDICARE:

B. “Medicare Benefit Plans” means Benefit Plans that replace, either partially or in their entirety, the original Medicare coverage (Medicare Part A and Medicare Part B) issued to beneficiaries by CMS,

but does not include Medicare Advantage Private Fee-For-Service plans and Medicare Advantage Medical Savings Account plans.

C. “AmeriChoice Medicare Benefit Plans” means Medicare Benefit Plans that include a reference to “Personal Care Plus” on the face of the valid identification card of any Customer eligible for and enrolled in such Benefit Plans.

D. “Evercare/SecureHorizons Medicare Benefit Plans” means Medicare Benefit Plans that are not AmeriChoice Medicare Benefit Plans.

MEDICAID AND OTHER STATE PROGRAMS:

E. “Medicaid Benefit Plans” means Benefit Plans that offer coverage to beneficiaries of a program authorized by Title XIX of the federal Social Security Act, and jointly financed by the federal and state governments and administered by the state.

F. “Other Governmental Benefit Plans” are Benefit Plans, other than Medicaid, SCHIP (also known as CHIP), or other Benefit Plans already identified in this Appendix 2, that are funded wholly or substantially by a state government or a subdivision of a state (such as a city or county), and for which eligibility is:

- i. based on the income and/or assets of the Customer or the Customer’s family, and
- ii. not based on whether the Customer or a member of the Customer’s family is or was an employee of the government.

G. “State Children’s Health Insurance Program” (“SCHIP” or “CHIP”) is the name of a program authorized by Title XXI of the federal Social Security Act, and jointly financed by the federal and state governments and administered by the state.

Additional Manuals Appendix

For services rendered to Customers enrolled in certain Benefit Plans or entities under common ownership with Carrier that may be included under this Agreement, Medical Group will be subject to additional requirements described in or made available to Medical Group through one or more additional provider manuals ("Additional Manuals"). When the Agreement refers to Protocols or Payment Policies it is also referring to the Additional Manuals. The Additional Manuals will be made available to Medical Group on a designated website or upon request. In the event of any conflict between the "UnitedHealthcare Physician, Health Care Professional, Facility and Ancillary Provider Administrative Guide" or other UnitedHealthcare Protocols and Payment Policies, and any Additional Manual, in connection with any matter pertaining to Customers enrolled in the Benefit Plans or entities under common ownership with Carrier to which the Additional Manual applies, that Additional Manual will govern, unless applicable statutes and regulations dictate otherwise. Carrier may make changes to the Protocols and Payment Policies subject to this Appendix in accordance with the provisions of the Agreement relating to Protocol and Payment Policy changes. The Benefit Plans or entities under common ownership with Carrier, names of the Additional Manuals, and name of the website to view and download the manuals, when applicable, are set forth in the table below. Carrier will notify Medical Group of any changes in the location of the Additional Manuals. Medical Group may request a copy of the Additional Manual.

Type of Benefit Plan	Description of Applicable Additional Manual	Website
Benefit Plans issued or administered by UnitedHealthcare of Washington, Inc., PacifiCare Life Assurance Company, and PacifiCare Life and Health Insurance Company	PacifiCare Provider Policy and Procedure Manual	www.pacificare.com
Healthy Options Benefit Plans	Washington Healthy Options Physician, Health Care Professional, Facility and Ancillary Administrative Guide	www.unitedhealthcare-washington.com
Washington Medicaid Benefit Plans	Washington Medicaid Physician, Health Care Professional, Facility and Ancillary Administrative Guide	www.unitedhealthcare-washington.com
Washington CHIP Benefit Plans	Washington CHIP Physician, Health Care Professional, Facility and Ancillary Administrative Guide	www.unitedhealthcare-washington.com

Appendix 3 - All Payer

All Payer Fee Information Document: WA 95404/95405

Unless another Appendix 3 to this Agreement applies specifically to a particular Benefit Plan as it covers a particular Customer, the provisions of this Appendix 3 apply to Covered Services rendered by Medical Group to Customers covered by Benefit Plans sponsored, issued or administered by all Payers.

Appendix 3 - Options PPO

Options PPO Fee Information Document: WA 95404/95405

The provisions of this Appendix 3 apply to Covered Services rendered by Medical Group to Customers covered by Benefit Plans marketed under the name "Options PPO."

Appendix 3 – Payment Appendix – Medicare Advantage

Medicare Advantage Fee Information Document: WA NZ2/NZ3

The provisions of this Payment Appendix apply to services rendered by Medical Group to Customers covered by Medicare Advantage Benefit Plans, as described in this Agreement.

Appendix 3-

Washington Medicaid and CHIP

Applicability

This Appendix applies to Covered Services rendered by Medical Group to Customers covered under the following types of Benefit Plans, as described in this Agreement:

- Washington Medicaid and CHIP Benefit Contracts

SECTION 1

Payment for Covered Services

1.1 Payment. Medical Group's contract rates for Covered Services are 100 % of the amounts as published in the Washington Medicaid fee schedule by the applicable state agency. This fee schedule is the "primary fee source" for contract rates. The actual payment amount is also subject to matters described in this Agreement, such as the Payment Policies.

For certain CPT codes, Carrier may use a higher amount than the contract rate described above. If Carrier does pay a higher amount, then at a future date Carrier may revert to the contract rate described above. In these circumstances, Carrier will not attempt to collect as overpayments amounts paid higher than the contract rate.

In the event that a specific dollar amount is not published in a fee source, then the contract rate for Covered Services will be priced at 40 % of eligible charges. Eligible charges are Customary Charges for Covered Services.

If an applicable state or federal program is available to provide items or payment directly to Medical Group for specific Covered Services for Customers subject to this Appendix that would otherwise be payable under this Appendix, the applicable program will apply and not this Appendix. (For example, the Vaccines For Children program currently provides vaccines free of charge, and therefore no amount will be payable under this Appendix for vaccines within the Vaccines For Children program. However, the administration of such vaccine is payable under this Appendix, because payment is not provided to physicians under the Vaccines For Children program for vaccine administration.)

Unless specifically indicated otherwise, amounts listed in this fee schedule represent global fees and may be subject to reductions based on appropriate modifiers (for example, professional and technical modifiers). Any co-payment, deductible or coinsurance that the Customer is responsible to pay under the Customer's Benefit Plan will be subtracted from the listed amount in determining the amount to be paid by Payer. This information is subject to the confidentiality provisions of this Agreement. A sample of the fee schedule is attached.

1.2 Routine Maintenance. Carrier routinely updates this fee schedule in response to changes published by the primary fee source, such as fee amount changes. Carrier will use reasonable commercial efforts to implement the fee schedule changes in its systems within 90 days after final publication and make them effective in our system on the effective date of the change by the primary fee source. However, claims already processed prior to the change being implemented by Carrier will not be reprocessed unless otherwise required by law.

Carrier also routinely updates this fee schedule in response to coding changes as described in this Agreement. When implementing coding updates, Carrier will apply the same percentages as set forth above in section 1.1 and the then current value of the published code to determine the contract rate. Carrier will use reasonable commercial efforts to implement such changes within 90 days from the date of publication. However, claims already processed prior to the change being implemented by Carrier will not be reprocessed unless otherwise required by law.

1.3 Medicaid Agency Payment Changes. If the Medicaid agency changes the manner in which it reimburses or changes the Medicaid primary fee source such that Carrier is required to make significant programming or platform changes in order to implement the Medicaid agency changes, Carrier will make commercially reasonable efforts to implement the Medicaid agency changes, within a reasonable time frame, from the date the change is published in the Medicaid agency's official correspondence to Carrier or is otherwise formally communicated by the Medicaid agency to Carrier. Medical Group agrees that, in such case, it shall accept the current payment as set forth in this Appendix until such a time as Carrier can implement the Medicaid agency change. At such time as Carrier is able to implement the change, Carrier will communicate the change and the effective date of the change via a copy of a new payment appendix. From that effective date forward, the contract rate will be calculated based on the new Medicaid agency payment.

If Carrier is unable, through commercially reasonable efforts, to incorporate the Medicaid agency payment changes in their entirety, Carrier will so notify Medical Group within ninety (90) days from the UHC/MGA.09.09.WA Parties are bound to confidentiality provisions stated under Section 10.9 of this Agreement

date the change is published in the Medicaid agency's official correspondence to Carrier, or otherwise formally communicated by the Medicaid agency. The parties shall then negotiate in good faith for a period of up to sixty (60) days to amend the Agreement to replace this Appendix with a new Appendix and stated effective date for the new contract rates. If the parties have not reached an agreement upon such an amendment within the aforementioned sixty (60) day period, either party may initiate Dispute Resolution according to this Agreement.

For reference purposes, the following fee schedules will be used to pay claims under this agreement:

PRODUCTS:	FEE SCHEDULES:	PROVIDER DESCRIPTION
All Payer	WA 95404/95405	Custom Fee Schedule: \$80 CF/Radiology at CF
Commercial - Options PPO	WA 95404/95405	Custom Fee Schedule: \$80 CF/Radiology at CF
Medicare	WA NZ2/NZ3	
Medicaid as described in appendix 2	SEE MEDICAID PAYMENT APPENDIX	SEE MEDICAID PAYMENT APPENDIX

Payment Appendix **Fee Information Document** **Fee Schedule Specifications: as of 01/02/2013** **Report Date: 01/02/2013**

Fee Schedule ID: WA 95404 - NonFacility

Linked Fee Schedule ID: WA 95405 - Facility

Type Of Service	Primary Fee Source	Pricing Level
EVALUATION & MANAGEMENT	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
EVALUATION & MANAGEMENT - NEONATAL	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
EVALUATION & MANAGEMENT - PREVENTIVE	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - INTEGUMENTARY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - MUSCULOSKELETAL	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - RESPIRATORY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - CARDIOVASCULAR	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - HEMIC & LYMPHATIC	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - MEDIASTINUM & DIAPHRAGM	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - DIGESTIVE	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - URINARY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - MALE GENITAL	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - FEMALE GENITAL	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - MATERNITY & DELIVERY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - ENDOCRINE	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - NERVOUS	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - EYE & OCULAR ADNEXA	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
SURGERY - AUDITORY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - BONE DENSITY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - CT	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - MAMMOGRAPHY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - MRI	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - MRA	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - NUCLEAR MEDICINE	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - PET SCANS	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - RADIATION THERAPY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
RADIOLOGY - ULTRASOUND	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
LAB - PATHOLOGY	2011 CMS RBRVS Carrier Locality (0000000)	60.000%
OFFICE LAB	2011 CMS Clinical Lab Schedule National Limit	60.000%
CLINICAL LABORATORY	2011 CMS Clinical Lab Schedule National Limit	42.000%
MEDICINE - OPHTHALMOLOGY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - CARDIOVASCULAR	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	2011 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	2011 CMS RBRVS Carrier Locality (0000000)	100.000%
MEDICINE - ENTERAL FORMULA	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - OTHER	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - IMMUNIZATION ADMINISTRATION	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
MEDICINE - CHEMO ADMIN	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
OBSTETRICS - GLOBAL	2011 CMS RBRVS Carrier Locality (0000000)	235.458%
IMMUNIZATIONS	UHC Immunization Fee Schedule	120.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES -- ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	2011 CMS DME Floor	60.000%
DME & SUPPLIES - RESPIRATORY	2011 CMS DME Floor	60.000%
DME & SUPPLIES - ORTHOTICS	2011 CMS DME Floor	60.000%
DME & SUPPLIES - PROSTHETICS	2011 CMS DME Floor	60.000%
DME & SUPPLIES - OSTOMY	2011 CMS DME Floor	60.000%
AMBULANCE	2011 CMS Ambulance Schedule - Urban (0000000)	235.458%

Default Percent of Eligible Charges: 40.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service: Site of Service applies, CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 80.00

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Last Routine Maintenance Update: 01-01-2013

Fixed Fees: 36415 - \$3.00 36416 - \$3.00 87804 - \$14.00 V5242 - \$2500.00 V5243 - \$2500.00 V5244 - \$2500.00 V5245 - \$2500.00 V5246 - \$2500.00 V5247 - \$2500.00 V5248 - \$5000.00 V5249 - \$5000.00 V5250 - \$5000.00 V5251 - \$5000.00 V5252 - \$5000.00 V5253 - \$5000.00 V5254 - \$2500.00 V5255 - \$2500.00 V5256 - \$2500.00 V5257 - \$2500.00 V5258 - \$5000.00 V5259 - \$5000.00 V5260 - \$5000.00 V5261 - \$5000.00 V5262 - \$2500.00 V5263 - \$5000.00

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. No payments will be made for any CMS additional compensation programs, including without limitation incentive or bonus payment programs. Please remember that this information is subject to the confidentiality provisions of this agreement.

Payment Appendix Fee Information Document

Representative Fee Schedule Sample for Hematology Oncology: as of 01/02/2013
Report Date: 01/02/2013

Fee Schedule ID: WA 95404 - NonFacility

Linked Fee Schedule ID: WA 95405 - Facility

CPT/HCPCS	Modifier	CPT/HCPCS Description	Type of Service	Place of Service	Fee Amount
77014	00	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 452.01
77014	26	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 100.00
77014	TC	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 352.01
77300	00	BASIC RADIATION	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 164.80
77301	00	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 4916.81
77301	26	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 945.60
77301	TC	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 3971.21
77413	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 552.01
77414	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 617.61
77418	00	INTENS MOD TX DE	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 1224.00
77421	00	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 259.19
77421	26	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 45.61
77421	TC	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 213.58
77427	00	RADIATION TREATM	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 424.79
78815	00	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 4489.97
78815	26	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 291.19
78815	TC	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 4198.78
96365	00	IV INFUSION THER	MEDICINE - OTHER	NonFacility	\$ 167.20
96367	00	IV INFUSION THER	MEDICINE - OTHER	NonFacility	\$ 77.61
96375	00	THERAPEUTIC INJE	MEDICINE - OTHER	NonFacility	\$ 53.59
96413	00	CHEMOTX ADMN IV	MEDICINE - CHEMO ADMIN	NonFacility	\$ 344.80
96417	00	CHEMOTX ADMN IV	MEDICINE - CHEMO ADMIN	NonFacility	\$ 170.40
99205	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 463.99
99213	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 162.40
99214	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 240.80
99215	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 323.99
99223	00	INITIAL HOSPITAL	EVALUATION & MANAGEMENT	NonFacility	\$ 456.81
99232	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	NonFacility	\$ 163.20
99233	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	NonFacility	\$ 234.40
J0681	00	INJECTION DARBEP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 3.32
J0885	00	INJECTION EPOETI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 9.87
J1300	00	INJECTION ECUZLZ	INJECTABLES/OTHER DRUGS	NonFacility	\$ 195.46
J1441	00	INJECTION FILGRA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 426.87
J2353	00	INJ OCTREOTIDE D	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 127.00
J2469	00	INJECTION PALONO	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 19.02
J2505	00	INJECTION PEGFIL	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 2638.89
J3487	00	INJECTION ZOEDR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 224.10
J9025	00	INJECTION AZACIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 5.52
J9033	00	INJECTION, BENDA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 20.61
J9035	00	INJECTION BEVACI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 62.32
J9041	00	INJECTION BORTEZ	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 43.47
J9055	00	INJECTION CETUXI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 51.09
J9171	00	INJECTION DOCECTA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 6.69
J9201	00	INJECTION, GEMCI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 12.25
J9263	00	INJECTION OXALIP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 9.25
J9264	00	INJECTION PACITR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 9.59
J9305	00	INJECTION PEMETR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 56.51
J9310	00	INJECTION, RITUX	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 659.72
J9355	00	INJECTION, TRAST	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 76.40
J9395	00	INJECTION FULVES	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 87.04

Default Percent of Eligible Charges: 40.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service Price Differential: Site of Service applies, CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 80.00

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Last Routine Maintenance Update: 01-01-2013

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. No payments will be made for any CMS additional compensation programs, including without limitation incentive or bonus payment programs. Please remember that this information is subject to the confidentiality provisions of this agreement.

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Payment Appendix
All Payer Fee Information Document
Fee Schedule Specifications: as of 05/18/2012
Report Date: 05/18/2012

Fee Schedule ID: WA NZ2 - NonFacility and Linked Fee Schedule ID: WA NZ3 - Facility

Type Of Service	Primary Fee Source (Carrier Locality)	Pricing Level
EVALUATION & MANAGEMENT	Current Year CMS RBRVS (0240299)	100.000%
EVALUATION & MANAGEMENT - NEONATAL	Current Year CMS RBRVS (0240299)	100.000%
EVALUATION & MANAGEMENT - PREVENTIVE	Current Year CMS RBRVS (0240299)	100.000%
EVALUATION & MANAGEMENT - NURSING FACILITY SVCS	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - INTEGUMENTARY	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - MUSCULOSKELETAL	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - RESPIRATORY	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - CARDIOVASCULAR	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - HEMIC & LYMPHATIC	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - MEDIASTINUM & DIAPHRAGM	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - DIGESTIVE	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - URINARY	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - MALE GENITAL	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - FEMALE GENITAL	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - MATERNITY & DELIVERY	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - ENDOCRINE	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - NERVOUS	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - EYE & OCULAR ADNEXA	Current Year CMS RBRVS (0240299)	100.000%
SURGERY - AUDITORY	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - BONE DENSITY	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - CT	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - MAMMOGRAPHY	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - MRI	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - MRA	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - NUCLEAR MEDICINE	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - PET SCANS	Current Year CMS RBRVS (0240299)	100.000%
RADIATION THERAPY	Current Year CMS RBRVS (0240299)	100.000%
RADIOLOGY - ULTRASOUND	Current Year CMS RBRVS (0240299)	100.000%
LAB - PATHOLOGY	Current Year CMS RBRVS (0240299)	100.000%
OFFICE LAB	Current Year CMS Clinical Lab Schedule - State Specific WA	100.000%
CLINICAL LABORATORY	Current Year CMS Clinical Lab Schedule - State Specific WA	100.000%
MEDICINE - OPHTHALMOLOGY	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - CARDIOVASCULAR	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - ALLERGY & CLINICAL IMMUNOLOGY	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - CHIROPRACTIC MANIPULATIVE TREATMENT	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - PHYSICAL MED AND REHAB - MODALITIES	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - PHYSICAL MED AND REHAB - THERAPIES&OTHER	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - ENTERAL FORMULA	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - OTHER	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - IMMUNIZATION ADMINISTRATION	Current Year CMS RBRVS (0240299)	100.000%
MEDICINE - CHEMO ADMIN	Current Year CMS RBRVS (0240299)	100.000%
OBSTETRICS - GLOBAL	Current Year CMS RBRVS (0240299)	100.000%
IMMUNIZATIONS	CMS Drug Pricing	100.000%
INJECTABLES/OTHER DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - ONCOLOGY/THERAPEUTIC CHEMO DRUGS	CMS Drug Pricing	100.000%
INJECTABLES - IVIG	CMS Drug Pricing	100.000%
INJECTABLES-SALINE & DEXTROSE SOLUTIONS	CMS Drug Pricing	100.000%
DME & SUPPLIES	Current Year CMS DME State WA	100.000%
DME & SUPPLIES - RESPIRATORY	Current Year CMS DME State WA	100.000%
DME & SUPPLIES - ORTHOTICS	Current Year CMS DME State WA	100.000%
DME & SUPPLIES - PROSTHETICS	Current Year CMS DME State WA	100.000%
DME & SUPPLIES - OSTOMY	Current Year CMS DME State WA	100.000%
AMBULANCE	Current Year CMS Ambulance Schedule - Urban (0240299)	100.000%

Default Percent of Eligible Charges: 35.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service: Site of Service applies, CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 21.30

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Future Payment Terms: The Anesthesia Conversion Factor will be updated to remain current with CMS.

Last Routine Maintenance Update: 04-01-2012

Fixed Fees: V5242 - \$2500.00 V5243 - \$2500.00 V5244 - \$2500.00 V5245 - \$2500.00 V5246 - \$2500.00 V5247 - \$2500.00 V5248 - \$5000.00 V5249 - \$5000.00 V5250 - \$5000.00 V5251 - \$5000.00 V5252 - \$5000.00 V5253 - \$5000.00 V5254 - \$2500.00 V5255 - \$2500.00 V5256 - \$2500.00 V5257 - \$2500.00 V5258 - \$5000.00 V5259 - \$5000.00

Unless specifically indicated otherwise, Fee Amounts listed in the fee schedule represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. Please remember that this information is subject to the confidentiality provisions of this agreement.

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Payment Appendix
All Payer Fee Information Document
Fee Schedule Specifications: as of 05/18/2012
Report Date: 05/18/2012

Fee Schedule ID: WA NZ2 - NonFacility and Linked Fee Schedule ID: WA NZ3 - Facility

V5260 - \$5000.00 V5261 - \$5000.00 V5262 - \$2500.00 V5263 - \$5000.00

Unless specifically indicated otherwise, Fee Amounts listed in the fee schedule represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. Please remember that this information is subject to the confidentiality provisions of this agreement.

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Payment Appendix Fee Information Document

Representative Fee Schedule Sample for Hematology Oncology: as of 01/02/2013
Report Date: 01/02/2013

Fee Schedule ID: WA 95404 - NonFacility

Linked Fee Schedule ID: WA 95405 - Facility

CPT/HCPCS	Modifier	CPT/HCPCS Description	Type of Service	Place of Service	Fee Amount
77014	00	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 452.01
77014	26	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 100.00
77014	TC	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 352.01
77300	00	BASIC RADIATION	RADIOLOGY - RADIATION THERAPY	Facility	\$ 164.80
77301	00	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 4916.81
77301	26	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 945.60
77301	TC	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 3971.21
77413	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	Facility	\$ 552.01
77414	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	Facility	\$ 617.61
77418	00	INTENS MOD TX DE	RADIOLOGY - RADIATION THERAPY	Facility	\$ 1224.00
77421	00	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 259.19
77421	26	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 45.61
77421	TC	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 213.58
77427	00	RADIATION TREATM	RADIOLOGY - RADIATION THERAPY	Facility	\$ 424.79
78815	00	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 4489.97
78815	26	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 291.19
78815	TC	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 4198.78
96365	00	IV INFUSION THER	MEDICINE - OTHER	Facility	\$ 167.20
96367	00	IV INFUSION THER	MEDICINE - OTHER	Facility	\$ 77.61
96375	00	THERAPEUTIC INJE	MEDICINE - OTHER	Facility	\$ 53.59
96413	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	Facility	\$ 344.80
96417	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	Facility	\$ 170.40
99205	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 382.41
99213	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 116.01
99214	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 178.41
99215	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 252.01
99223	00	INITIAL HOSPITAL	EVALUATION & MANAGEMENT	Facility	\$ 456.81
99232	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	Facility	\$ 163.20
99233	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	Facility	\$ 234.40
J0881	00	INJECTION DARBEP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 3.32
J0885	00	INJECTION EPOETI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 9.87
J1300	00	INJECTION ECUZIL	INJECTABLES/OTHER DRUGS	Facility	\$ 195.46
J1441	00	INJECTION FILGRA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 426.87
J2353	00	INJ OCTREOTIDE D	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 127.00
J2469	00	INJECTION PALONO	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 19.02
J2505	00	INJECTION PEGFIL	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 2638.89
J3487	00	INJECTION ZOLEDR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 224.10
J9025	00	INJECTION AZACIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 5.52
J9033	00	INJECTION, BENDA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 20.61
J9035	00	INJECTION BEVACI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 62.32
J9041	00	INJECTION BORTEZ	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 43.47
J9055	00	INJECTION CETUXI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 51.09
J9171	00	INJECTION DOCECTA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 6.69
J9201	00	INJECTION, GEMCI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 12.25
J9263	00	INJECTION OXALIP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 9.25
J9264	00	INJECTION PACLIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 9.59
J9305	00	INJECTION PEMETR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 56.51
J9310	00	INJECTION, RITUX	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 659.72
J9355	00	INJECTION, TRAST	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 76.40
J9385	00	INJECTION FULVES	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 87.04

Default Percent of Eligible Charges: 40.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 80.00

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Last Routine Maintenance Update: 01-01-2013

Fee Amounts listed in the fee schedule are all-inclusive, including without limitation any applicable taxes. Unless specifically indicated otherwise, Fee Amounts represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. No payments will be made for any CMS additional compensation programs, including without limitation incentive or bonus payment programs. Please remember that this information is subject to the confidentiality provisions of this agreement.

Confidential and Proprietary Not for Distribution to Third Parties

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Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. In the event that any of United's claims systems cannot administer a 15 minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$[(\text{Value of 15 minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}]$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } [(\$60.00 / 15) * 10 = \$40.00]$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.unitedhealthcareonline.com.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

In the event that any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

CPT/HCPCS: A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: In the event that a Fee Basis amount is not sourced by either a primary or alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in nonfacility Places of Service.

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.

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Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

Place of Service: The facility or nonfacility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the primary or alternate Fee Source.

RVU: Relative Value Unit as published by CMS. United uses the RVU that is used by CMS. For example, if CMS uses a transitional RVU, then United will as well.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes within the same Type of Service category.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

In the event that any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as published by CMS).

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The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment decision while the patient is in the office.

A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

Section 2. Alternate Fee Sources

In the event the Primary Fee Source contains no published Fee Basis amount, alternate (or "gap fill") Fee Sources may be used to supply the Fee Basis amount for deriving the Fee Amount. For example, if a new CPT/HCPCS code has been created within the Type of Service category of codes described above, and CMS has not yet established an RBRVS value for that code, we use one of the Fee Sources that exist within the industry to fill that gap, such as but not limited to Ingenix Essential RBRVS. For that CPT/HCPCS code, we adopt the RBRVS value established by the gap-fill Fee Source, and determine the Fee Amount for that CPT/HCPCS code by applying to the gap-fill RBRVS the same Conversion Factor and Pricing Level that we apply to the CMS RBRVS for those CPT/HCPCS codes that have CMS RBRVS values. At such time in the future as CMS publishes its own RBRVS value for that CPT/HCPCS code, we would begin using the Primary Fee Source, CMS, to derive the Fee Amount for that code and no longer use the alternate Fee Source.

More information about all of our Fee Sources can be located at:

- Centers for Medicare and Medicaid Services (CMS) RBRVS and Fee Schedules: www.cms.hhs.gov
- Centers for Disease Control and Prevention (CDC) Private Sector Selling Price: www.cdc.gov/vaccines/programs/vfc/cdc-vac-price-list.htm
- Thomson Reuters Red Book: www.micromedex.com
- RJ Health Systems: www.reimbursementcodes.com
- Ingenix Essential RBRVS: www.ingenixonline.com
- American Society of Anesthesiologists: www.asahq.org

Section 3. Routine Updates

Routine updates occur when United mechanically incorporates revised information created by the Fee Source, and as described below, to update the Fee Amounts calculated in accordance with this Fee Information Document. United routinely updates its fee schedule: (1) to stay current with applicable coding practices; (2) in response to price changes for immunizations and injectable medications; and (3) to remain in compliance with HIPAA requirements. United will not generally attempt to communicate routine updates of this nature.

The types of routine updates, and their respective effective dates, are described below.

a. Annual Changes to Relative Value Units, Conversion Factors, or Flat Rate Fees

This fee schedule follows a "stated year" construction methodology. The 2011 RVU, the 2011 Conversion Factor, and the 2011 flat rate fees (non-RVU based fees such as DME fees) will be locked in as the basis for deriving Fee Amounts. Therefore, the annual publication of RVUs and Conversion Factors by CMS may affect this fee schedule. Generally, any RVU, Conversion Factor, or flat rate fee changes published in subsequent years by the Primary Fee Sources will not be reflected in this fee schedule except, for example, to add Fee Amounts for new codes or to replace alternate Fee Basis amounts. United will use reasonable commercial efforts to implement the updates in its systems on or before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law. In the event that CMS does not publish a complete set of Fee Basis amounts for a specific code (for example: Global, -TC, and -26 fees) and the code contains a status code of "C" (indicating the code is carrier priced), United will use reasonable commercial efforts to establish Fee Amounts for all modifiers associated with the code based on fee information available and published by the local fiscal intermediary and by fiscal intermediaries from other locations.

b. Quarterly Updates in Response to Changes Published by Primary Fee Sources

United updates its fee schedule in response to changes published by Primary Fee Sources as a result of additions, deletions, and changes to CPT codes by the AMA or HCPCS codes by CMS and any subsequent changes to CMS' annual update. United updates its fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of the updates described in this subsection b. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that

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quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection b. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection b. will be effective no later than October 1. In the event that CMS does not publish a complete set of Fee Basis amounts for a specific code (for example: Global, -TC, and -26 fees) and the code contains a status code of "C" (indicating the code is carrier priced), United will use reasonable commercial efforts to establish Fee Amounts for all modifiers associated with the code based on fee information available and published by the local fiscal intermediary and by fiscal intermediaries from other locations.

However, in the event that the code source has expired a CPT/HCPCS code and replaced it with a Replacement Code, United will crosswalk the fee from the Expired Code to its Replacement Code as further described below:

Based on information published by the code source (AMA Current Procedural Terminology and The HCPCS Level II), when one Expired Code is replaced by one Replacement Code, United will apply the Expired Code's Fee Amount to the Replacement Code; provided, however, if the Expired Code's Fee Amount was determined by an alternate Fee Source and a Primary Fee Source becomes available, the Replacement Code's Fee Amount will be determined using the Primary Fee Source.

Based on information published by the code source (AMA Current Procedural Terminology and The HCPCS Level II) and United's claims data, when several Expired Codes that are always done in conjunction with each other are replaced by one Replacement Code, United will apply the sum of these Expired Code's Fee Amounts to the Replacement Code; provided, however, if the Expired Code's Fee Amount was determined by an alternate Fee Source and a Primary Fee Source becomes available, the Replacement Code's Fee Amount will be determined using the Primary Fee Source.

The following types of codes are not included in our direct crosswalk methodology as described above:

- Temporary HCPCS codes, such as G, K, Q, and S codes
- Temporary CPT codes, such as Category III codes
- Informational codes, such as CPT Category II codes
- HCPC-C Codes, which are only used by hospitals
- Codes categorized as immunizations and injectables

If any types of codes not currently listed in the exclusions above are developed in the future, United reserves the right to make a crosswalk determination at that time.

c. Price Changes for Immunizations and Injectables

United routinely updates the Fee Amounts in response to price changes for immunizations and injectables published by the Fee Sources. In addition, United's Executive Drug Pricing Forum (EDPF) meets on a quarterly basis to review and evaluate the drug prices that will be used in each quarterly update. The EDPF may address topics including pricing for emerging drugs, anticipated manufacturer price changes, and special circumstances (for example, H1N1 vaccine). Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug. These Fee Amount updates will be effective as described below.

For Immunizations, United applies the UHC Immunization Fee Schedule. The Centers for Disease Control and Prevention Private Sector Selling Price (CDC PSSP) is the Primary Fee Source used to obtain the Fee Basis amounts. In the event that more than one Fee Basis amount is published by the CDC PSSP for a specific CPT/HCPCS code, an average of the published amounts will be used.

More information about the UHC Immunization Fee Schedule can be located at: www.unitedhealthcareonline.com >> Claims & Payments > Fee Schedule Lookup > Related Links "UHC Immunization Fee Schedule"

The effective date of updates under this subsection c. will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c. will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c. will be effective no later than October 1.

d. Other Updates

United reserves the right, but not the obligation, to perform other updates as may be necessary to remain consistent with a Primary Fee Source. United also will perform other updates as may be required by applicable law from time to time. United will use reasonable commercial efforts to implement the updates in its systems on or

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before the later of (i) 90 days after the effective date of any modification made by the Fee Source or (ii) 90 days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). United will make the updates effective in its system on the effective date of the change by the Fee Source. However, claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

For More Information

United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement or you may use our fee schedule look-up function on the web at: www.unitedhealthcareonline.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.

Medical Group Participation Agreement Appendix

This Appendix to the Medical Group Participation Agreement (the "Appendix") is entered into and made part of the Medical Group Participation Agreement between UnitedHealthcare Insurance Company, PacifiCare Life and Health Insurance Company, PacifiCare Life Assurance Company, and UnitedHealthcare of Washington, Inc., contracting on behalf of each entity individually, (collectively referred to as "Carrier") and RadiantCare Radiation Oncology LLC ("Medical Group").

The provisions in this Appendix labeled "The following provision applies to Washington regulated plans," apply to Benefit Plans sponsored, issued or administered by or accessed through Carrier to the extent such products are subject to regulation under Washington laws; and for which Washington laws control. The requirements in this Appendix, however, do not apply to the extent they are preempted by the Medicare Modernization Act or other applicable law.

The provisions in this Appendix labeled "The following provision applies to plans not regulated by Washington law" apply to benefit plans sponsored, issued or administered by or accessed through Carrier to the extent such products are not subject to regulation under Washington laws.

The provisions in this Appendix labeled "The following provision applies to all plans" apply to all products or benefit plans sponsored, issued or administered by or accessed through Carrier.

Carrier and Medical Group each agree to be bound by the terms and conditions contained in this Appendix. In the event of a conflict or inconsistency between this Appendix and any term or condition contained in the Agreement, this Appendix shall control.

This Appendix will deemed to be updated to incorporate any changes to the laws and regulations referenced herein, including any changes to definitions referenced herein, effective as of the date of such changes.

Unless otherwise defined in this Appendix, all capitalized terms contained in the Appendix shall be defined as set forth in the Agreement.

The following provision applies to Washington regulated plans:

Section 2.1 of the Agreement is deleted in its entirety and replaced with the following:

2.1 Representations and Warranties of Medical Group. Medical Group, by virtue of its execution and delivery of this Agreement, represents and warrants as follows:

(a) Medical Group is a duly organized and validly existing legal entity in good standing under the laws of its jurisdiction of organization.

(b) Medical Group has all requisite corporate power and authority to conduct its business as presently conducted, and to execute, deliver and perform its obligations under this Agreement. The execution, delivery and performance of this Agreement by Medical Group have been duly and validly authorized by all action necessary under its organizational documents and applicable corporate law.

(c) The execution, delivery and performance of this Agreement by Medical Group do not and will not violate or conflict with (i) the organizational documents of Medical Group, (ii) any material agreement or instrument to which Medical Group is a party or by which Medical Group or any material part of its property is bound, or (iii) applicable law. Medical Group has the unqualified authority to bind,

and does bind, itself and Medical Group Professionals to all of the terms and conditions of this Agreement, including any Appendices, Attachments and Exhibits, as applicable.

(d) Medical Group has obtained and holds all registrations, permits, licenses, and other approvals and consents, and has made all filings, that it is required to obtain from or make with all governmental entities under applicable law in order to conduct its business as presently conducted and to enter into and perform its obligations under this Agreement.

(e) Medical Group has been given an opportunity to review the Protocols and Payment Policies and agrees that it will follow the terms and conditions of the Protocols and that claims under this Agreement will be paid in accordance with the Payment Policies. See the Additional Manuals Appendix for additional information regarding the Protocols and Payment Policies applicable to Customers enrolled in certain Benefit Plans.

(f) Each submission of a claim by Medical Group pursuant to this Agreement shall be deemed to constitute the representation and warranty by it to Carrier that (i) the representations and warranties of it set forth in this section 2.1 and elsewhere in this Agreement are true and correct as of the date the claim is submitted, (ii) it has complied with the requirements of this Agreement with respect to the Covered Services involved and the submission of such claim, (iii) the charge amount set forth on the claim is the Customary Charge and (iv) the claim is a valid claim.

The following provision applies to Washington regulated plans:

Section 3.2 of the Agreement is deleted in its entirety and replaced with the following:

3.2 Payers and Benefit Plan types. Carrier may allow Payers to access Medical Group's services under this Agreement for the Benefit Plan types described in Appendix 2. Appendix 2 may be modified by Carrier upon 60 days written or electronic notice.

The following provision applies to Washington regulated plans:

Section 3.5 of the Agreement is deleted in its entirety and replaced with the following:

3.5 Health Care. Medical Group acknowledges that this Agreement and Customer Benefit Plans do not dictate the health care provided by Medical Group or Medical Group Professionals, or govern Medical Group's or Medical Group Professional's determination of what care to provide its patients, even if those patients are Customers. The decision regarding what care is to be provided remains with Medical Group Professionals and with Customers, and not with Carrier or any Payer.

Notwithstanding any other provision of law, Carrier, when subject to the jurisdiction of the state of Washington as a health carrier, may not prohibit directly or indirectly Customers from freely contracting at any time to obtain any health care services outside the health care plan on any terms or conditions Customer chooses. Nothing in this section shall be construed to bind Carrier for any services delivered outside the health plan.

The following provision applies to Washington regulated plans:

Section 3.6 of the Agreement is deleted in its entirety and replaced with the following:

3.6 Communication with Customers. Nothing in this Agreement is intended to limit Medical Group's or Medical Group Professional's right or ability to communicate fully with a Customer regarding the Customer's health condition and treatment options. Medical Group and Medical Group Professionals are free to discuss all treatment options without regard to whether or not a given option is a Covered Service. Medical Group and Medical Group Professionals are free to discuss with a Customer any financial

incentives Medical Group may have under this Agreement, including describing at a general level the payment methodologies contained in this Agreement. Carrier shall not prohibit, discourage, or penalize a provider otherwise practicing in compliance with the law from advocating on behalf of a Customer.

The following provision applies to Washington regulated plans:

Section 5.2 of the Agreement is deleted in its entirety and replaced with the following:

5.2 Nondiscrimination. Medical Group will not discriminate against any patient, with regard to quality of service or accessibility of services, on the basis that the patient is a Customer. Medical Group will not require a Customer to pay a "membership fee" or other fee in order to access Medical Group for Covered Services (except for co-payments, coinsurance and/or deductibles provided for under Customer's Benefit Plan) and will not discriminate against any Customer based on the failure to pay such a fee.

Medical Group shall treat all Customers equally, regardless of whether the Customer's enrollment is through a private purchaser or a publicly funded program such as Medicare or Medicaid.

The following provision applies to Washington regulated plans:

Section 5.4 of the Agreement is deleted in its entirety and replaced with the following:

5.4 Cooperation with Protocols. Medical Group will cooperate with and comply with Carrier's and Payers' Protocols subject to any applicable Regulatory Appendix. The Protocols include but are not limited to all of the following:

1. Medical Group will use reasonable commercial efforts to direct Customers only to other providers that participate in Carrier's network, except as otherwise authorized by Carrier or Payer.
2. If the Customer's Benefit Plan requires the Customer to receive certain Covered Services from or upon referral by a primary care physician, all referral physicians must adhere to the following additional protocols when those Covered Services are provided:
 - a. Notify Customer's primary care physician of referrals to other participating or non-participating providers.
 - b. Covered Services must be provided pursuant to the terms and limitations of the referral notification issued by or on behalf of the Customer's primary care physician.
 - c. If the Medical Group Physician providing the Covered Services is a referral physician, the Medical Group Physician must also notify the Customer's primary care physician of all admissions in accordance with the required time frames.
3. Medical Group will provide notification for certain Covered Services, accept and return telephone calls from Carrier staff, and respond to Carrier requests for clinical information, as required by Carrier or Payer as described in the Protocols.

The Protocols will be made available to Medical Group online or upon request. Some or all Protocols also may be disseminated in the form of an administrative manual or guide or in other communications.

Pursuant to WAC § 284-43-320(4), Carrier shall notify Medical Group of its responsibilities with respect to Carrier's applicable administrative protocols, policies and programs, including but not limited to payment terms, utilization review, quality assessment and improvement programs, credentialing, grievance procedures, data reporting requirements, confidentiality requirements and any applicable

federal or state requirements. Medical Group shall comply with the Carrier's protocols, policies and programs. Medical Group hereby acknowledges that it has been given the opportunity prior to contracting to access the Carrier's Administrative Guide at no cost to the Medical Group by way of Carrier's website or by hardcopy upon request to the Carrier. Therefore, Medical Group has had the opportunity to learn of its responsibilities under the Administrative Guide. The Administrative Guide describes, among other things, administrative and operational protocols and procedures, such as claims submission, and clinical submission requirements.

Carrier may change the Protocols from time to time. Carrier will use reasonable commercial efforts to inform Medical Group at least 30 days in advance of any material changes to the Protocols. Carrier may implement changes in the Protocols without Medical Group's consent if such change is applicable to all or substantially all of the medical groups in Carrier's network located in the same state as Medical Group and that practice the same specialty as Medical Group. Notwithstanding anything in this Agreement to the contrary, Medical Group must be given reasonable notice of not less than 60 days of any Protocol changes that affect Medical Group compensation and that affect health care service delivery, unless changes to federal or state law or regulations make such advance notice impossible in which case notice shall be provided as soon as possible. Subject to any termination and continuity of care provisions of the Agreement, Medical Group may terminate the Agreement without penalty if Medical Group does not agree with such changes. Changes to the Protocols proposed by Carrier to be applicable to Medical Group that are not applicable to all or substantially all of the medical groups in Carrier's network located in the same state as Medical Group and that practice the same specialty as Medical Group are subject to the terms of section 10.2 of this Agreement that are applicable to amendments. No change to the Agreement may be made retroactive without the express consent of Medical Group.

The following provision applies to Washington regulated plans:

Section 5.9 of the Agreement is deleted in its entirety and replaced with the following:

5.9 Maintenance of and Access to Records. Medical Group will maintain adequate medical, financial and administrative records related to Covered Services rendered by Medical Group under this Agreement, including claims records, for at least 6 years following the end of the calendar year during which the Covered Services are provided, unless a longer retention period is required by applicable law.

Medical Group will provide access to these records as follows:

- i) to Carrier or its designees, in connection with Carrier's utilization management/ care management, quality assurance and improvement and for claims payment and other administrative obligations, including reviewing Medical Group's compliance with the terms and provisions of this Agreement and appropriate billing practice. Medical Group will provide access during ordinary business hours within fourteen days after a request is made, except in cases of a Carrier audit involving a fraud investigation or the health and safety of a Customer (in which case, access shall be given within 48 hours after the request) or of an expedited Customer appeal or grievance (in which case, access will be given so as to enable Carrier to reasonably meet the timelines for determining the appeal or grievance); and
- ii) to agencies of the government, in accordance with applicable law, to the extent such access is necessary to comply with regulatory requirements applicable to Medical Group, Carrier, or Payers.

Medical Group shall be required to make health records available to appropriate state and federal authorities involved in assessing the quality of care or investigating grievances or complaints from Customers subject to applicable state and federal laws related to the confidentiality of medical or health records.

Medical Group will cooperate with Carrier on a timely basis in connection with any such audit including, among other things, in the scheduling of and participation in an audit exit interview within 30 days of Carrier's request.

Upon invoice from Medical Group, Carrier will pay for copies of records requested by Carrier in cases where Carrier requests the records more than once and the records are requested for some other purpose than claims processing, coverage determinations, or other routine health benefits administration. Payment will be made at a rate of \$0.25 cents per page, not to exceed a total of \$25.00 per record, unless a different rate is specified under state law.

To the extent the Agreement allows Carrier the right to audit Medical Group's billing records, Medical Group shall have the right to audit Carrier's denial of Medical Group's claims under the same terms and conditions the Agreement sets forth for Carrier's audit of Medical Group's billing records.

The following provision applies to Washington regulated plans:

Section 5.13 of the Agreement is deleted in its entirety and replaced with the following:

5.13 Employees and subcontractors. Medical Group will assure that its employees, affiliates and any individuals or entities subcontracted by Medical Group to render services in connection with this Agreement adhere to the requirements of this Agreement and the Washington Administrative Code. The use of employees, affiliates or subcontractors to render services in connection with this Agreement will not limit Medical Group's obligations and accountability under this Agreement with regard to such services.

For laboratory services, Medical Group will only be reimbursed for services that Medical Group is certified through the Clinical Laboratory Improvement Amendments (CLIA) to perform, and Medical Group must not bill Customers for any laboratory services for which Medical Group lacks CLIA certification.

The following provision applies to all plans:

Section 7.1 of the Agreement is deleted in its entirety and replaced with the following:

7.1 Form and content of claims. Medical Group must submit claims for Covered Services in a manner and format prescribed by Carrier, as further described in the Protocols. Unless otherwise directed by Carrier, Medical Group shall submit claims using current CMS 1500 or UB04 or successor forms for paper claims and HIPAA standard professional or institutional claim formats for electronic claims, as applicable, with applicable coding including, but not limited to, ICD, CPT, Revenue and HCPCS coding.

Medical Group will submit claims only for services performed by Medical Group or Medical Group staff. Pass through billing is not payable under this Agreement.

Carrier hereby acknowledges and agrees that, in order to offer complete care to Carrier's Customers, Medical Group shall procure professional services of oncologists employed by RadiantCare Physicians PLLC, a Washington professional limited liability company ("RadiantCare Physicians"), which professional services Medical Group cannot and does not provide. Medical Group shall submit a single global bill to the Carrier, which shall include all charges for both the technical services provided by Provider and the professional services rendered by RadiantCare Physicians. All payments of such bills shall be made to Medical Group. Upon receipt of payment of global bill from the Carrier, Provider shall remit payment to RadiantCare Physicians for the professional services rendered.

The following provision applies to Washington regulated plans:

Section 7.4 of the Agreement is deleted in its entirety and replaced with the following:

7.4 Payment of Claims. Payer will pay claims for Covered Services according to the lesser of Medical Group's Customary Charge or the applicable fee schedule (as further described in Appendix 3 to this Agreement), and in accordance with Payment Policies.

Carrier or Payer, as applicable, shall pay or deny claims in accordance with the claims payment standards contained in WAC 284-43-321 which are set forth below.

- (a) (i) For Covered Services provided to Customers, Carrier or Payer shall pay Medical Group as soon as practical but subject to the following minimum standards:
 - (A) Ninety-five percent of the monthly volume of clean claims shall be paid within thirty days of receipt by Carrier or agent of Carrier; and
 - (B) Ninety-five percent of the monthly volume of all claims shall be paid or denied within sixty days of receipt by Carrier or agent of Carrier, except as agreed to in writing by the parties on a claim-by-claim basis.
- (ii) The receipt date of a claim is the date Carrier or its agent receives either written or electronic notice of the claim.
- (iii) Carrier shall establish a reasonable method for confirming receipt of claims and responding to Medical Group inquiries about claims.
- (iv) If Carrier or Payer fails to pay claims within the standard established under subsection (a) of this section, Carrier or Payer shall pay interest on undenied and unpaid clean claims more than sixty-one days old until Carrier or Payer meets the standard under subsection (a) of this section. Interest shall be assessed at the rate of one percent per month, and shall be calculated monthly as simple interest prorated for any portion of a month. Carrier or Payer shall add the interest payable to the amount of the unpaid claim without the necessity of Medical Group submitting an additional claim. Any interest paid under this section shall not be applied by Carrier or Payer to a Customer's deductible, copayment, coinsurance, or any similar obligation of the Customer.
- (v) When Carrier or Payer issues payment in either Medical Group and the Customer names, Carrier or Payer shall make claim checks payable in the name of Medical Group first and the Customer second.
- (b) For purposes of this section, "clean claim" means a claim that has no defect or impropriety, including any lack of any required substantiating documentation, or particular circumstances requiring special treatment that prevents timely payments from being made on the claim under this section.
- (c) Denial of a claim must be communicated to Medical Group and must include the specific reason why the claim was denied. If the denial is based upon medical necessity or similar grounds, then Carrier or Payer upon request of Medical Group must also promptly disclose the supporting basis for the decision. For example, Carrier or Payer must describe how the claim failed to meet medical necessity guidelines.
- (d) Carrier or Payer shall be responsible for ensuring that any person acting on behalf of or at the direction of Carrier or Payer or acting pursuant to Carrier or Payer standards or requirements complies with these billing and claim payment standards.

- (e) These standards do not apply to claims about which there is substantial evidence of fraud or misrepresentation by Medical Group or Customers, or instances where Carrier or Payer has not been granted reasonable access to information under Medical Group's control.
- (f) Medical Group, Carrier and Payer are not required to comply with this section if the failure to comply is occasioned by any act of God, bankruptcy, act of a governmental authority responding to an act of God or other emergency, or the result of a strike, lockout, or other labor dispute.

Claims for Covered Services subject to coordination of benefits will be paid in accordance with the Customer's Benefit Plan and applicable state and federal law.

Carrier or Payer shall not unreasonably delay payment of a claim by reason of the application of a coordination of benefits provision. Carrier or Payer shall make payment of a claim involving the application of a coordination of benefit provision 30 days after receipt of the claim. When payment of a claim is necessarily delayed for reasons other than the application of a coordination of benefits provision, investigation of other plan coverage shall be conducted concurrently, so as to create no further delay in the ultimate payment of benefits. If Carrier or Payer is required by the above stated time limit to make payment as the primary plan, and Carrier or Payer is not the primary plan, Carrier or Payer may exercise its rights under the Agreement's "right of recovery" provision to recover any excess payments made thereby.

The obligation for payment under this Agreement is primarily that of Payer, and not that of Carrier unless Carrier is the Payer.

Ordinarily, fee amounts listed in Appendix 3 are based upon primary fee sources. Carrier reserves the right to use gap-fill fee sources where primary fee sources are not available.

Carrier routinely updates its fee schedule in response to additions, deletions, and changes to CPT codes by the American Medical Association, price changes for immunizations and injectable medications, and in response to similar changes (additions and revisions) to other service coding and reporting conventions that are widely used in the health care industry, such as those maintained by the Centers for Medicare and Medicaid Services (for example HCPCS, etc.). Ordinarily, Carrier's fee schedule is updated using similar methodologies for similar services. Carrier will not generally attempt to communicate routine maintenance of this nature and will generally implement updates within 90 days from the date of publication.

Carrier will give Medical Group 90 days written or electronic notice of non-routine fee schedule changes which will substantially alter the overall methodology or reimbursement level of the fee schedule. In the event such changes will reduce Medical Group's overall reimbursement under this Agreement, Medical Group may terminate this Agreement by giving 60 days written notice to Carrier, provided that the notice is given by Medical Group within 30 days after the notice of the fee schedule change.

Carrier will make its Payment Policies available to Medical Group online or upon request. Carrier may change its Payment Policies from time to time.

Notwithstanding anything in this Agreement to the contrary, Medical Group must be given reasonable notice of not less than sixty days of changes to any Payment Policies that affect Medical Group compensation and that affect health care service delivery, unless changes to federal or state law or regulations make such advance notice impossible in which case notice shall be provided as soon as possible. Subject to any termination and continuity of care provisions of the Agreement, Medical Group

may terminate the Agreement without penalty if Medical Group does not agree with such changes. No change to the Agreement may be made retroactive without the express consent of Medical Group.

The following provision applies to Washington regulated plans:

Section 7.5 of the Agreement is deleted in its entirety and replaced with the following:

7.5 Denial of Claims for Not Following Protocols, Not Filing Timely, or Lack of Medical Necessity.

Payment may be denied in whole or in part, or an administrative sanction may be imposed, if Medical Group does not comply with a Protocol or does not file a timely claim under section 7.3 of this Agreement. Payment may also be denied for services provided that are determined by Carrier to be medically unnecessary, and Medical Group may not bill the Customer for such services unless the Customer has, with knowledge of Carrier's determination of a lack of medical necessity, agreed in writing to be responsible for payment of those charges.

In the event that payment of a claim is denied for lack of notification or for untimely filing, the denial will be reversed if Medical Group appeals within 12 months after the date of denial and can show all of the following:

- i) that, at the time the Protocols required notification or at the time the claim was due, Medical Group did not know and was unable to reasonably determine that the patient was a Customer,
- ii) that Medical Group took reasonable steps to learn that the patient was a Customer, and
- iii) that Medical Group promptly provided notification, or filed the claim, after learning that the patient was a Customer.

In the event Carrier offers a Benefit Plan that is governed by RCW 48.43.525 or WAC 284-43-410(3), Carrier will comply with the applicable retrospective claim denial requirements as set forth in RCW 48.43.525 and WAC 284-43-410(3) and will supersede the other provisions of this section 7.5, "Denial of claims for not following Protocols or not filing timely, or Lack of Medical Necessity," of the Agreement.

The following provision applies to Washington regulated plans:

Section 7.8 of the Agreement is deleted in its entirety and replaced with the following:

7.8 Customer "Hold Harmless." Medical Group will not bill or collect payment from the Customer, or seek to impose a lien, for the difference between the amount paid under this Agreement and Medical Group's billed charge or Customary Charge, or for any amounts denied or not paid under this Agreement due to:

- i) Medical Group's failure to comply with the Protocols,
- ii) Medical Group's failure to file a timely claim,
- iii) Payer's Payment Policies,
- iv) inaccurate or incorrect claim processing,
- v) insolvency or other failure by Payer to maintain its obligation to fund claims payments, if Payer is Carrier, or is an entity required by applicable law to assure that its Customers not be billed in such circumstances, or
- vi) a denial based on medical necessity or prior authorization, except as permitted under section 7.5.

This obligation to refrain from billing Customers applies even in those cases in which Medical Group believes that Carrier or Payer has made an incorrect determination. In such cases, Medical Group may

pursue remedies under this Agreement against Carrier or Payer, as applicable, but must still hold the Customer harmless.

Medical Group also hereby agrees that in no event, including, but not limited to nonpayment by Payer, Payer's insolvency or breach of this Agreement shall Medical Group bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against a Customer or person acting on the Customer's behalf, other than Carrier, for services provided pursuant to this Agreement. This provision shall not prohibit collection of any copayments, deductibles or coinsurance for which the Customer is responsible under the Benefit Plan, which have not otherwise been paid by a primary or secondary carrier in accordance with regulatory standards for coordination of benefits, from Customers in accordance with the terms of the Customer's Benefit Plan. Medical Group agrees, in the event of the Payer's insolvency, to continue to provide the services promised in this Agreement to Customers for the duration of the period for which premiums on behalf of the Customer were paid or until the Customer's discharge from inpatient facilities, whichever time is greater. Notwithstanding any other provision of this Agreement, nothing in this Agreement shall be construed to modify the rights and benefits contained in the Customer's Benefit Plan. Medical Group may not bill the Customer for Covered Services (except for any copayments, deductibles or coinsurance for which the Customer is responsible under the Benefit Plan) where Payer denies payments because the Medical Group has failed to comply with the terms or conditions of this Agreement. Medical Group further agrees: (a) that the provisions of this section shall survive termination of this Agreement regardless of the cause giving rise to termination and shall be construed to be for the benefit of the Customers and (b) that this section supersedes any oral or written or written contrary agreement now existing or hereafter entered into between Medical Group and Customers or persons acting on the Customer's behalf.

If Medical Group contracts with other health care providers who agree to provide Covered Services to Customers with the expectation of receiving payment directly or indirectly from Payer, such providers must also agree to abide by this section.

If Medical Group willfully collects or attempts to collect an amount from a Customer knowing that collection to be in violation of this Agreement, such collection or attempts to collect shall constitute a class C felony under Washington Statutes.

This section 7.8 and section 7.7 will survive the termination of this Agreement, with regard to Covered Services rendered prior to when the termination takes effect.

The following provision applies to Washington regulated plans:

Section 7.10 of the Agreement is deleted in its entirety and replaced with the following:

7.10 Correction of Overpayments or Underpayments of Claims. In the event that either Party believes that a claim has not been paid correctly, or that funds were paid beyond or outside of what is provided for under this Agreement, either party may seek correction of the payment by giving the other party notice, within 12 months after the payment was initially made, that it believes the payment was made incorrectly.

The 12 month limit will not apply to recovery of overpayments in either of the following circumstances:

- i) the overpayment resulted from fraud or abuse by or on behalf of Medical Group, or
- ii) Carrier's ability to discover the overpayment during the 12 month period was hindered by Medical Group's failure to provide full and timely cooperation with an audit by Carrier.

Medical Group will repay overpayments within 30 days of notice of the overpayment. Medical Group will promptly report any credit balance that it maintains with regard to any claim overpayment under this

Agreement, and will return such overpayment to Carrier within 30 days after posting it as a credit balance.

Medical Group agrees that recovery of overpayments may be accomplished by offsets against future payments.

Subsections (a) and (b) below apply in the event Carrier offers a Benefit Plan that is governed by RCW 48.43, and only if Medical Group is a "health care provider" as that term is defined in RCW 48.43.005.

- (a) Carrier Requests for Refund. Except in cases of fraud or as otherwise provided in this section and RCW 48.43, (i) All requests for refunds must be made by Carrier in writing within twenty-four (24) months after the date that the payment was made; (ii) Requests for refunds that relate to coordination of benefits must be made within thirty (30) months after the date that the payment was made; and (iii) Carrier may not request a contested refund to be paid any sooner than six (6) months after receipt of the request. Requests for refunds must specify why Carrier believes Medical Group owes the refund. Requests for a refund that involve coordination of benefits must include the name and mailing address of the entity that has primary responsibility for payment of the claim. If Medical Group fails to contest the request in writing to Carrier within thirty (30) days of its receipt, the request is deemed accepted and the refund must be paid.

Carrier may at any time request a refund from Medical Group of a payment previously made to satisfy a claim if: (A) A third party, including a government entity, is found responsible for satisfaction of the claim as a consequence of liability imposed by law, such as tort liability; and (B) Carrier is unable to recover directly from the third party because the third party has either already paid or will pay Medical Group for the health services covered by the claim.

- (b) Medical Group Requests for Additional Payment. Except in cases of fraud or as otherwise provided in this section and RCW 48.43, (i) All requests by Medical Group for additional payment to satisfy a claim must be made in writing within twenty-four (24) months after the date that the claim was denied or initial payment intended to satisfy the claim was made; (ii) Requests for additional payments to satisfy claims that relate to coordination of benefits must be made within thirty (30) months after the date that the payment was made; and (iii) Medical Group may not request a contested refund to be paid any sooner than six (6) months after receipt of the request. Requests for additional payments must specify why Medical Group believes Payer owes the additional payment. Requests for additional payment that involve coordination of benefits must include the name and mailing address of the entity that has disclaimed responsibility for payment of the claim.

The following provision applies to Washington regulated plans:

Section 7.11 of the Agreement is deleted in its entirety and replaced with the following:

7.11 Claims Payment Issues Arising from Departure of Medical Group Professionals from Medical Group. In the event a Medical Group Professional departs from Medical Group and uncertainty arises as to whether Medical Group or some other entity is entitled to receive payment for certain services rendered by such former Medical Group Professional, the parties will cooperate with each other in good faith in an attempt to resolve the situation appropriately.

In the event that Medical Group's failure to give timely notice under section 5.7 (iv) of this Agreement resulted in claims payments being made incorrectly to Medical Group, Medical Group shall promptly call the situation to Carrier's attention and return such payments to Carrier. In the event Medical Group fails

to do so, Carrier may hold Medical Group liable for any attorneys fees, costs, or administrative expenses incurred by Carrier as a result.

In the event that Medical Group and some other entity assert a right to payment for the same service rendered by the former Medical Group Professional, Carrier shall pay Medical Group according to the terms of the Agreement.

The following provision applies to Washington regulated plans:

Article VIII of the Agreement is deleted in its entirety and replaced with the following:

Article VIII
Dispute Resolution

The parties will work together in good faith to resolve any and all disputes between them (hereinafter referred to as "Disputes") including but not limited to all questions of the existence, validity, scope or termination of the Agreement or any term thereof. If the Dispute pertains to a matter which is generally administered by certain Carrier procedures, such as a credentialing or quality improvement plan, the policies and procedures set forth in that plan must be fully exhausted by Medical Group before Medical Group may invoke any rights to dispute resolution under this Article VIII.

If the parties are unable to resolve any such Dispute within 60 days following the date one party sent written notice of the Dispute to the other party, and if either party wishes to pursue the Dispute, it shall thereafter be submitted to nonbinding mediation in accordance with the Commercial Dispute Procedures of the American Arbitration Association, as they may be amended from time to time (see <http://www.adr.org>). Unless otherwise agreed to in writing by the parties, the party wishing to pursue the Dispute must initiate the mediation within one year after the date on which notice of the Dispute was given or shall be deemed to have waived its right to pursue the dispute in any forum. If following notice of the Dispute the parties mutually desire to resolve the Dispute through binding arbitration, nothing in this Agreement shall be construed as precluding them from so agreeing.

Further, Carrier shall provide reasonable means that allow Medical Group, if aggrieved by actions of Carrier, to be heard after Medical Group submits a written request to Carrier. If Carrier fails to grant or reject a request within thirty days after it has been made, Medical Group may proceed as if the complaint had been rejected; except that in the case of a billing dispute that has been timely-made by Medical Group as required under section 7.10, Carrier shall render a decision within sixty days of receipt of Medical Group's complaint. A complaint that has been rejected by Carrier or any dispute Carrier has with Medical Group relating to the terms of this Agreement may be submitted to nonbinding mediation.

Any mediation proceeding under this Agreement shall be conducted in King County, WA. The mediator may construe or interpret but shall not vary or ignore the terms of this Agreement and shall be bound by controlling law. The mediator shall have no authority to award punitive, exemplary, indirect or special damages, except in connection with a statutory claim that explicitly provides for such relief. The parties acknowledge that because this Agreement affects interstate commerce the Federal Arbitration Act applies.

If the parties are unable to resolve a Dispute through nonbinding mediation, and if either party wishes to pursue the Dispute, such party may commence litigation proceedings. The parties agree that if litigation is used to resolve a Dispute, neither party shall be entitled to an award of special, indirect, punitive or exemplary damages, except in connection with a statutory claim that explicitly provides for such relief. In no event shall any litigation be commenced more than one year after the date on which notice of the Dispute was given.

The parties expressly intend that any Dispute relating to the business relationship between them be resolved on an individual basis so that no other dispute with any third party(ies) may be consolidated or joined with the Dispute. The parties agree that any ruling allowing class action litigation or arbitration or requiring consolidated litigation or arbitration involving any third party(ies) would be contrary to their intent and would require immediate judicial review of such ruling.

In the event that any portion of this Article or any part of this Agreement is deemed to be unlawful, invalid or unenforceable, such unlawfulness, invalidity or unenforceability shall not serve to invalidate any other part of this Article or Agreement.

In the event a party wishes to terminate this Agreement based on an assertion of uncured material breach, and the other party disputes whether grounds for such a termination exist, while such Dispute remains pending, the termination for breach will not take effect.

This Article VIII governs any dispute between the parties arising before or after execution of this Agreement, and shall survive any termination of this Agreement.

The following provision applies to Washington regulated plans:

Section 9.2 of the Agreement is deleted in its entirety and replaced with the following:

9.2 Termination. This Agreement may be terminated under any of the following circumstances:

- i) by mutual written agreement of the parties;
- ii) by either party without cause, upon at least 90 days prior written notice, effective at the end of the initial term or effective at the end of any renewal term;
- iii) by either party upon 60 days written notice in the event of a material breach of this Agreement by the other party, except that such a termination will not take effect if the breach is cured within 60 days after notice of the termination; moreover, such termination may be deferred as further described in Article VIII of this Agreement;
- iv) by either party upon 10 days written notice in the event the other party loses licensure or other governmental authorization necessary to perform this Agreement, or fails to have insurance as required under section 5.6 or section 6.2 of this Agreement; or
- v) by Medical Group, as described in section 7.4 of this Agreement in the event of a non-routine fee schedule change.

Pursuant to WAC 284-43-320(7), in the event the Agreement is terminated for any reason, Carrier or Payer shall make a good faith effort to give written notice of such impending termination to Customers who are seen by Medical Group on a regular basis, no later than 15 working days prior to the effective date of termination.

The following provision applies to Washington regulated plans:

Section 9.3 of the Agreement is deleted in its entirety and replaced with the following:

9.3 Ongoing Services to Certain Customers After Termination Takes Effect. In the event a Customer is receiving any of the Covered Services listed below, as of the date the termination takes effect, Medical Group will continue to render those Covered Services to that Customer, and this Agreement will continue to apply to those Covered Services, after the termination takes effect, for the length of time indicated below:

Inpatient Covered Services	30 days or until discharge, whichever comes first
Pregnancy, Third Trimester – Low Risk	Through postpartum follow up visit
Pregnancy, First, Second or Third Trimester – Moderate Risk and High Risk	Through postpartum follow up visit
Non-Surgical Cancer Treatment	30 days or a complete cycle of radiation or chemotherapy, whichever is greater
End Stage Kidney Disease and Dialysis	30 days
Symptomatic AIDS undergoing active treatment	30 days
Circumstances where Payer is required by applicable law to provide transition coverage of services rendered by Medical Group after Medical Group leaves the provider network accessed by Payer.	As applicable

Notwithstanding the prior paragraph, in the event this Agreement is terminated and in the event Medical Group is a primary care physician and Medical Group is rendering Covered Services to Customers, Medical Group agrees to continue providing such Covered Services pursuant to the terms and conditions of this Agreement (a) for at least sixty (60) days following termination; and (b) in the case of group Benefit Plans, until the end of the next open enrollment period.

The following provision applies to Washington regulated plans:

Section 10.2 of the Agreement is deleted in its entirety and replaced with the following:

10.2 Amendment. Carrier can amend this Agreement or any of the appendices on 90 days written or electronic notice by sending Medical Group a copy of the amendment, unless changes to federal or state law or regulations make such advance notice impossible, in which case notice shall be provided as soon as possible. Medical Group's signature is not required to make the amendment effective. If the amendment results in changes that affect compensation and that affect health care service delivery, then subject to any termination and continuity of care provisions of the Agreement, Medical Group may terminate the Agreement without penalty if Medical Group does not agree with such changes. No change to the Agreement may be made retroactive without the express consent of Medical Group.



Payment Appendix All Payer Fee Information Document

Representative Fee Schedule Sample for Hematology Oncology: as of 05/18/2012
Report Date: 05/18/2012

Fee Schedule ID: WA NZ2 - NonFacility

Linked Fee Schedule ID: WA NZ3 - Facility

CPT/HCPCS	Modifier	CPT/HCPCS Description	Type of Service	Place of Service	Fee Amount
77014	00	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 144.76
77014	26	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 41.82
77014	TC	CT GUIDANCE RADI	RADIOLOGY - CT	NonFacility	\$ 102.94
77300	00	BASIC RADIATION	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 68.09
77301	00	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 2002.05
77301	26	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 393.17
77301	TC	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 1608.88
77413	00	RADJ DLVR 3/> AR	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 243.63
77414	00	RADJ DLVR 3/> AR	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 273.79
77418	00	INTENS MOD TX DE	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 481.50
77421	00	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 87.23
77421	26	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 18.73
77421	TC	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 68.50
77427	00	RADIATION TREATM	RADIOLOGY - RADIATION THERAPY	NonFacility	\$ 176.36
78815	00	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 709.06
78815	26	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 119.47
78815	TC	PET IMAGING CT A	RADIOLOGY - PET SCANS	NonFacility	\$ 589.59
96365	00	IV INFUSION THER	MEDICINE - OTHER	NonFacility	\$ 73.13
96367	00	IV INFUSION THER	MEDICINE - OTHER	NonFacility	\$ 32.59
96375	00	THERAPEUTIC INJE	MEDICINE - OTHER	NonFacility	\$ 22.64
96413	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	NonFacility	\$ 139.82
96417	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	NonFacility	\$ 71.75
99205	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 199.17
99213	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 70.55
99214	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 104.28
99215	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	NonFacility	\$ 139.99
99223	00	INITIAL HOSPITAL	EVALUATION & MANAGEMENT	NonFacility	\$ 194.65
99232	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	NonFacility	\$ 69.84
99233	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	NonFacility	\$ 99.84
J0881	00	INJECTION DARBEP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 3.21
J0885	00	INJECTION EPOETI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 9.62
J1300	00	INJECTION ECULIZ	INJECTABLES/OTHER DRUGS	NonFacility	\$ 194.11
J1441	00	INJECTION FILGRA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 408.13
J2353	00	INJ OCTREOTIDE D	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 121.59
J2469	00	INJECTION PALONO	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 18.47
J2505	00	INJECTION PEGFIL	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 2754.07
J3487	00	INJECTION ZOLED	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 226.83
J9025	00	INJECTION AZACIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 5.26
J9033	00	INJECTION, BENDA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 18.81
J9035	00	INJECTION BEVACI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 61.09
J9041	00	INJECTION BORTEZ	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 42.45
J9055	00	INJECTION CETUXI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 50.46
J9171	00	INJECTION DOCETA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 13.96
J9201	00	INJECTION, GEMCI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 51.19
J9263	00	INJECTION OXALIP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 10.26
J9264	00	INJECTION PACLIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 9.63
J9305	00	INJECTION PEMETR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 55.06
J9310	00	INJECTION, RITUX	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 626.92
J9355	00	INJECTION, TRAST	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 72.45
J9395	00	INJECTION FULVES	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	NonFacility	\$ 86.25

Default Percent of Eligible Charges: 35.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 21.30

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Future Payment Terms: The Anesthesia Conversion Factor will be updated to remain current with CMS.

Last Routine Maintenance Update: 04-01-2012

Unless specifically indicated otherwise, Fee Amounts listed in the fee schedule represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. Please remember that this information is subject to the confidentiality provisions of this agreement.

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Payment Appendix All Payer Fee Information Document

Representative Fee Schedule Sample for Hematology Oncology: as of 05/18/2012
Report Date: 05/18/2012

Fee Schedule ID: WA NZ2 - NonFacility

Linked Fee Schedule ID: WA NZ3 - Facility

CPT/HCPCS	Modifier	CPT/HCPCS Description	Type of Service	Place of Service	Fee Amount
77014	00	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 144.76
77014	26	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 41.82
77014	TC	CT GUIDANCE RADI	RADIOLOGY - CT	Facility	\$ 102.94
77300	00	BASIC RADIATION	RADIOLOGY - RADIATION THERAPY	Facility	\$ 68.09
77301	00	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 2002.05
77301	26	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 393.17
77301	TC	INTENS MOD RADIO	RADIOLOGY - RADIATION THERAPY	Facility	\$ 1608.88
77413	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	Facility	\$ 243.83
77414	00	RADJ DLVR 3/4 AR	RADIOLOGY - RADIATION THERAPY	Facility	\$ 273.79
77418	00	INTENS MOD TX DE	RADIOLOGY - RADIATION THERAPY	Facility	\$ 481.50
77421	00	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 87.23
77421	26	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 18.73
77421	TC	STRSC X-RAY GDN	RADIOLOGY - RADIATION THERAPY	Facility	\$ 68.50
77427	00	RADIATION TREATM	RADIOLOGY - RADIATION THERAPY	Facility	\$ 176.36
78815	00	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 709.06
78815	26	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 119.47
78815	TC	PET IMAGING CT A	RADIOLOGY - PET SCANS	Facility	\$ 589.59
96365	00	IV INFUSION THER	MEDICINE - OTHER	Facility	\$ 73.13
96367	00	IV INFUSION THER	MEDICINE - OTHER	Facility	\$ 32.59
96375	00	THERAPEUTIC INJE	MEDICINE - OTHER	Facility	\$ 22.64
96413	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	Facility	\$ 139.82
96417	00	CHEMOTX ADMIN IV	MEDICINE - CHEMO ADMIN	Facility	\$ 71.75
99205	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 161.97
99213	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 49.54
99214	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 76.03
99215	00	OFFICE OUTPATIENT	EVALUATION & MANAGEMENT	Facility	\$ 106.92
99223	00	INITIAL HOSPITAL	EVALUATION & MANAGEMENT	Facility	\$ 194.65
99232	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	Facility	\$ 69.64
99233	00	SBSQ HOSPITAL CA	EVALUATION & MANAGEMENT	Facility	\$ 99.84
J0881	00	INJECTION DARBEP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 3.21
J0885	00	INJECTION EPOETI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 9.62
J1300	00	INJECTION ECULIZ	INJECTABLES/OTHER DRUGS	Facility	\$ 194.11
J1441	00	INJECTION FILGRA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 408.13
J2353	00	INJ OCTREOTIDE D	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 121.59
J2469	00	INJECTION PALONO	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 18.47
J2505	00	INJECTION PEGFIL	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 2754.07
J3487	00	INJECTION ZOLEDR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 226.83
J9025	00	INJECTION AZACIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 5.26
J9033	00	INJECTION BENDA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 18.81
J9035	00	INJECTION BEVACI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 61.09
J9041	00	INJECTION BORTEZ	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 42.45
J9055	00	INJECTION CETUXI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 50.46
J9171	00	INJECTION DOCETA	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 13.96
J9201	00	INJECTION GEMCI	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 51.19
J9263	00	INJECTION OXALIP	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 10.26
J9264	00	INJECTION PACLIT	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 9.63
J9305	00	INJECTION PEMETR	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 55.06
J9310	00	INJECTION, RITUX	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 626.92
J9355	00	INJECTION, TRAST	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 72.45
J9395	00	INJECTION FULVES	INJECTABLES-ONCOLOGY/THERAPEUTIC CHEMO DRUGS	Facility	\$ 86.25

Default Percent of Eligible Charges: 35.00%

Professional/Technical Modifier Pricing: Fee Source-Based

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 = F)

Anesthesia Conversion Factor (Based on a 15 minute Anesthesia Time Unit Value): \$ 21.30

Calculation of Anesthesia Partial Units: Proration

Schedule Type: FFS

Future Payment Terms: The Anesthesia Conversion Factor will be updated to remain current with CMS.

Last Routine Maintenance Update: 04-01-2012

Unless specifically indicated otherwise, Fee Amounts listed in the fee schedule represent global fees and may be subject to reductions based on appropriate Modifier (for example, professional and technical modifiers). As used in the previous sentence, "global fees" refers to services billed without a Modifier, for which the Fee Amount includes both the professional component and the technical component. Any co-payment, deductible or coinsurance that the customer is responsible to pay under the customer's benefit contract will be subtracted from the listed Fee Amount in determining the amount to be paid by the payer. The actual payment amount is also subject to matters described in this agreement, such as the Payment Policies. Please remember that this information is subject to the confidentiality provisions of this agreement.

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Payment Appendix

All Payer Fee Information Document

Additional Information About This Fee Schedule

Fee Schedule ID: WA NZ2 - NonFacility

Linked Fee Schedule ID: WA NZ3 - Facility

Section 1. Definition of Terms

Unless otherwise defined in this document, capitalized terms will have the meanings ascribed to them in the Agreement.

AMA: American Medical Association located at: www.ama-assn.org.

Anesthesia Conversion Factor: The dollar amount that will be used in the calculation of time-based and non-time based Anesthesia Management fees in accordance with the Anesthesia Payment Policy. Unless specifically stated otherwise, the Anesthesia Conversion Factor indicated is fixed and will not change. The Anesthesia Conversion Factor is based on an anesthesia time unit value of 15 minutes. In the event that any of United's claims systems cannot administer a 15 minute anesthesia time unit value, then the Anesthesia Conversion Factor will be calculated as follows:

$$[(\text{Value of 15 minute Anesthesia Conversion Factor} / 15) * \text{anesthesia time unit value}]$$

For example, an Anesthesia Conversion Factor of \$60.00 (based on a 15-minute anesthesia time unit value) would be calculated to an Anesthesia Conversion Factor of \$40.00 (based on a 10-minute anesthesia time unit value).

$$\text{Example: } [(\$60.00 / 15) * 10 = \$40.00]$$

Anesthesia Management: The management of anesthesia services related to medical, surgical or scopic procedures, as described in the current Anesthesia Management Codes list attached to the Anesthesia Payment Policy located at www.unitedhealthcareonline.com.

Calculation of Anesthesia Partial Units:

Proration: Partial time units will be prorated and calculated to one decimal place rounded to the nearest tenth. For example, if the anesthesia time unit value is based on 15 minutes and if 17 minutes of actual time is submitted on a claim, then the 17 minutes will be divided by 15. The resulting figure of 1.1333 will be rounded to the nearest tenth and the total time units for the claim will be 1.1 time units.

In the event that any of United's claims systems cannot administer the calculation of partial units as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the Proration method described above.

CMS: Centers for Medicare and Medicaid Services located at: www.cms.hhs.gov.

Conversion Factor: A multiplier, expressed in dollars per relative value unit, which converts relative values into Fee Basis amounts.

CPT/HCPCS: A set of codes that describe procedures and services, including supplies and materials, performed or provided by physicians and other health care professionals. Each procedure or service is identified with a 5 digit code. The use of CPT/HCPCS simplifies the reporting of services.

CPT/HCPCS Description: The descriptor associated with each CPT/HCPCS code.

Default Percent of Eligible Charges: In the event that a Fee Basis amount is not sourced by either a primary or alternate Fee Source, such as services submitted using unlisted, unclassified or miscellaneous codes, the codes are subject to correct coding review and will be priced at the contracted percentage indicated within this document.

Expired Code: An existing CPT or HCPCS code that will be expired by the entity that published the code (for example, CMS or the AMA).

Fee Amount: The contract rate for each CPT/HCPCS. The calculation of the Fee Amount is impacted by a variety of factors explained within this document including, but not limited to, Professional/Technical Modifier Pricing, Carrier Locality, CMS year, Place of Service and Pricing Level.

Fee Basis: The amount published by the Fee Source upon which the Pricing Level will be applied to derive the Fee Amount.

Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule which supports the terms of the contractual agreement. This is the fee schedule for services performed in nonfacility Places of Service.

Fee Schedule Specifications: Documentation of the underlying calculation methodology and criteria used to derive the Fee Amounts contained within the fee schedule.



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Fee Schedule ID: WA NZ2 - NonFacility

Linked Fee Schedule ID: WA NZ3 - Facility

Fee Source: The primary or alternate entity or publication that is supplying the Fee Basis.

Fixed Fees: Fee Amounts that are set at amounts which do not change. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Future Payment Terms: The general description of any pricing terms which will be implemented on a scheduled future effective date.

Last Routine Maintenance Update: The effective date on which this fee schedule was most recently updated. Please refer to the Routine Maintenance section of this document for more information about Routine Maintenance updates.

Linked Fee Schedule ID: United's proprietary naming/numbering convention that is used to identify the specific fee schedule for each specific contractual agreement. This is the fee schedule for services performed in facility Places of Service.

Modifier: A Modifier provides the means to report or indicate that a service or procedure has been altered by some specific circumstance but not changed in its definition or code.

Place of Service: The facility or nonfacility setting in which the service is performed. This may also be referred to by CMS as Payment Type.

Pricing Level: The contracted percentage or amount that will be multiplied times the primary or alternate Fee Basis amount in order to derive the Fee Amount.

Primary Fee Source (Carrier Locality): The main Fee Source used to supply the Fee Basis amount for deriving the Fee Amount within each Type of Service category. For instance, if the Fee Amounts for a given category of codes are derived by applying a particular Pricing Level to the CMS Resource-Based Relative Value Scale (RBRVS), then CMS RBRVS is the Primary Fee Source. The Carrier Locality is designated to indicate the exact CMS geographic region upon which the Fee Amounts are based.

Professional/Technical Modifier Pricing: Fee Source-Based: Fee Amounts for Modifiers (for example, -TC or -26 Modifiers) are derived using the Fee Basis amount as published by the primary or alternate Fee Source.

RVU: Relative Value Unit as published by CMS.

Replacement Code: One or more new CPT or HCPCS codes that are the exact same services or descriptions and will replace one or more Expired Codes.

Report Date: The actual date that this document was produced.

Representative Fee Schedule Sample: A representative listing of the most commonly used CPT/HCPCS codes and fees, along with other relevant pricing information, for each specific Fee Schedule ID. The Fee Amounts listed are intended for pricing purposes only and are subject to other matters described in this Agreement, such as the Payment Policies.

Schedule Type: FFS: This is a fee-for-service fee schedule. Unless stated otherwise, the Fee Amount indicated will be used to calculate payment to you as further described within this document.

Site of Service Price Differential: Site of Service applies. CMS Assignment (ASC POS 24 =F): This fee schedule follows CMS guidelines for determining when services are priced at the facility or nonfacility fee schedule (with the exception of services performed at Ambulatory Surgery Centers, POS 24, which will be priced at the facility fee schedule). CMS guidelines can be located at: www.cms.hhs.gov.

In the event that any of United's claims systems cannot administer the calculation of Site of Service Differential pricing as indicated above, a different calculation method will be used until such time as the appropriate system enhancements can be programmed and implemented. That different calculation method will result in a Fee Amount that is no less than the Fee Amount that would apply under the method described above.

Type of Service: A general categorization of related CPT/HCPCS codes. Type of Service categories are intended to closely align with the CPT groupings in the Current Procedural Terminology code book (as published by the AMA) and the HCPCS groupings (as published by CMS).

The Office Lab Type of Service category represents those lab tests, as determined by United, in which the lab test result is necessary to make an informed treatment



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decision while the patient is in the office.

A partial or complete crosswalk mapping of CPT/HCPCS to Type of Service categories is available to you upon request.

Section 2. Alternate Fee Sources

In the event the Primary Fee Source contains no published Fee Basis amount, alternate (or "gap fill") Fee Sources may be used to supply the Fee Basis amount for deriving the Fee Amount. For example, if a new CPT/HCPCS code has been created within the Type of Service category of codes described above, and CMS has not yet established an RBRVS value for that code, we use one of the Fee Sources that exist within the industry to fill that gap, such as but not limited to Ingenix Essential RBRVS. For that CPT/HCPCS code, we adopt the RBRVS value established by the gap-fill Fee Source, and determine the Fee Amount for that CPT/HCPCS code by applying to the gap-fill RBRVS the same Conversion Factor and Pricing Level that we apply to the CMS RBRVS for those CPT/HCPCS codes that have CMS RBRVS values. At such time in the future as CMS publishes its own RBRVS value for that CPT/HCPCS code, we would begin using the Primary Fee Source, CMS, to derive the Fee Amount for that code and no longer use the alternate Fee Source.

More information about all of our Fee Sources can be located at:

- Centers for Medicare and Medicaid Services (CMS) RBRVS and Fee Schedules: www.cms.hhs.gov
- Centers for Disease Control and Prevention (CDC) Private Sector Selling Price: www.cdc.gov/vaccines/programs/vrk/cdc-vac-prs-c-list.htm
- Thomson Reuters Red Book: www.micromedex.com
- RJ Health Systems: www.reimbursementcodes.com
- Ingenix Essential RBRVS: www.ingenixonline.com
- American Society of Anesthesiologists: www.asahq.org

Section 3. Routine Maintenance

Except as further described within this document, United will use reasonable commercial efforts to update the amounts listed in this fee schedule as published by the Fee Source on or before the later of (a) ninety (90) days after the effective date of any modification made by the Fee Source or (b) ninety (90) days after the date on which the Fee Source initially places information regarding such modification in the public domain (for example, when CMS distributes program memoranda to providers). Claims already processed prior to the change being implemented by United will not be reprocessed unless otherwise required by law.

United routinely updates its fee schedule in an effort to stay current with coding practices widely used in the health care industry; in response to price changes for immunizations and injectable medications; and to remain in compliance with the intent of the contractual agreement. Routine maintenance occurs when United mechanically incorporates revised information created by a third party that is the Fee Source. United will not generally attempt to communicate routine maintenance of this nature and will use reasonable commercial efforts to implement updates as further described below. Providers may expect the following types of fee updates to this fee schedule:

a. Annual Changes to Relative Value Units, Conversion Factors, or Flat Rate Fees

The annual publication of RVUs and Conversion Factors by CMS may affect this fee schedule. This fee schedule follows a "current year" construction methodology; therefore, it is intended to remain current with RVU, Conversion Factor, and flat rate fee (non-RVU based fees such as DME fees) changes as the basis for deriving Fee Amounts. As such, the annual update changes published by CMS will similarly be reflected in this fee schedule for dates of service on and after the effective date published by CMS. Unless specifically stated otherwise, for those anesthesia services that are contracted on a time-based methodology, the Anesthesia Conversion Factor indicated within this document is fixed and will not change. Please refer to the Anesthesia Conversion Factor section above.

b. Price Changes for Immunizations and Injectable Medications

United routinely updates its fee schedule in response to price changes for immunizations and injectable medications published by the Fee Sources. The effective date of fee updates under this subsection b will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the next calendar quarter. For example, if final publication by the fee source is on April 10, the fee update under this subsection b will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection b will be effective no later than October 1. For purposes of this paragraph, the date of a claim is the date of service.

United's Executive Drug Pricing Forum (EDPF) meets on a quarterly basis to review and evaluate the drug prices that will be used in each quarterly update. Topics that the EDPF may address:



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- Emerging drugs pricing
- Anticipated or pending manufacturer price changes
- Special circumstance pricing (for example, H1N1 vaccine)

Based on supporting information provided by the drug manufacturer or the Fee Source, United's EDPF may elect to establish a Fee Amount or override a Fee Amount, as published by the Fee Source, in favor of a Fee Amount that is more appropriate and reasonable for a particular vaccine or drug.

c. CPT/HCPCS Code Updates

United routinely updates its fee schedule in response to additions, deletions, and changes to CPT codes by the AMA; HCPCS codes by CMS; CMS changes to its annual update; and in response to similar changes (additions and revisions) to other service coding and reporting conventions that are widely used in the health care industry. United updates our fee schedules for new CPT/HCPCS codes using the applicable Conversion Factor and Pricing Level of the original construction methodology along with the then-current RVU of the published CPT/HCPCS code. The effective date of fee updates under this subsection c will be no later than the first day of the next calendar quarter after final publication by the Fee Source, except that if that quarter begins less than 60 days after final publication, the effective date will be no later than the first day of the calendar quarter following the next calendar quarter. For example, if final publication by the Fee Source is on April 10, the fee update under this subsection c will be effective no later than July 1, and if final publication by the Fee Source is on June 10, the fee update under this subsection c will be effective no later than October 1. For purposes of this paragraph, the date of a claim is the date of service.

d. Carrier Priced Codes

In the event that CMS does not publish a complete set of Fee Basis amounts for a specific code (for example: Global, -TC, and -26 fees) and the code contains a status code of "C" (indicating the code is carrier priced), United will use reasonable commercial efforts to establish Fee Amounts for all modifiers associated with the code based on fee information available and published by the local fiscal intermediary and by fiscal intermediaries from other locations.

For More Information

United is committed to providing transparency related to our fee schedules. If you have questions about this fee schedule, please contact Network Management at the address and phone number on your contract or participation agreement or you may use our fee schedule look-up function on the web at: www.unitedhealthcareonline.com or contact our Voice Enabled Telephonic Self Service line at (877) 842-3210.

MEDICARE ADVANTAGE REGULATORY REQUIREMENTS APPENDIX

THIS MEDICARE ADVANTAGE REGULATORY REQUIREMENTS APPENDIX (this “Appendix”) supplements and is made part of the network participation agreement (the “Agreement”) between United and the physician or provider named in the Agreement (“Provider”).

SECTION 1 APPLICABILITY

This Appendix applies to the Covered Services Provider provides to Medicare Advantage Customers. In the event of a conflict between this Appendix and other appendices or any provision of the Agreement, the provisions of this Appendix shall control except: (1) with regard to Benefit Plans outside the scope of this Appendix; (2) as noted in Section 2 of this Appendix; or (3) as required by applicable law.

SECTION 2 DEFINITIONS

For purposes of this Appendix, the following terms shall have the meanings set forth below; provided, however, in the event any definition set forth in this Appendix is in conflict with any definition in the Agreement for the same or substantially similar term, the definition for such term in the Agreement shall control. All other capitalized terms not otherwise defined in this Appendix shall be as defined in the Agreement.

2.1 Benefit Plan: A certificate of coverage, summary plan description, or other document or agreement, whether delivered in paper, electronic, or other format, under which a Payer is obligated to provide coverage of Covered Services for a Customer. Benefit Plan may also be referred to as benefit contract, benefit document, plan, or other similar term under the Agreement.

2.2 CMS Contract: A contract between the Centers for Medicare & Medicaid Services (“CMS”) and a Medicare Advantage Organization for the provision of Medicare benefits pursuant to the Medicare Advantage Program under Title XVIII, Part C of the Social Security Act.

2.3 Cost Sharing: Those costs, if any, under a Benefit Plan that are the responsibility of the Customer, including deductibles, coinsurance, and copayments. Cost Sharing may also be referred to as patient expenses or other similar term under the Agreement.

2.4 Covered Service: A health care service or product for which a Customer is entitled to receive coverage from a Payer, pursuant to the terms of the Customer’s Benefit Plan with that Payer. A Covered Service may also be referred to as a health service or other similar term under the Agreement.

2.5 **Customer:** A person eligible and enrolled to receive coverage from a Payer for Covered Services. A Customer may also be referred to as an enrollee, member, patient, covered person, or other similar term under the Agreement.

2.6 **Dual Eligible Customer:** A Medicare Advantage Customer who is: (a) eligible for Medicaid; and (b) for whom the state is responsible for paying Medicare Part A and B Cost Sharing.

2.7 **Medicare Advantage Benefit Plans:** Benefit Plans sponsored, issued or administered by a Medicare Advantage Organization as part of the Medicare Advantage program or as part of the Medicare Advantage program together with the Prescription Drug program under Title XVIII, Part C and Part D, respectively, of the Social Security Act (as those program names may change from time to time).

2.8 **Medicare Advantage Customer or MA Customer:** A Customer eligible for and enrolled in a Medicare Advantage Benefit Plan in which Provider participates pursuant to the Agreement.

2.9 **Medicare Advantage Organization or MA Organization:** For purposes of this Appendix, MA Organization is either United or Payer.

2.10 **Payer:** An entity obligated to a Customer to provide reimbursement for Covered Services under the Customer's Benefit Plan and authorized by United to access Provider's services under the Agreement. A Payer may also be referred to as a payor, participating entity or other similar term under the Agreement.

2.11 **United:** UnitedHealthcare Insurance Company and/or one or more of its affiliates.

SECTION 3 PROVIDER REQUIREMENTS

3.1 **Data.** Provider shall submit to MA Organization all risk adjustment data as defined in 42 CFR 422.310(a), and other Medicare Advantage program-related information as may be requested by MA Organization, within the timeframes specified and in a form that meets Medicare Advantage program requirements. By submitting data to MA Organization, Provider represents to MA Organization, and upon MA Organization's request Provider shall certify in writing, that the data is accurate, complete, and truthful, based on Provider's best knowledge, information and belief.

3.2 **Policies.** Provider shall cooperate and comply with MA Organization's policies and procedures.

3.3 **Customer Protection.** Provider agrees that in no event, including but not limited to, non-payment by MA Organization or an intermediary, insolvency of MA Organization or an intermediary, or breach by United of the Agreement, shall Provider bill, charge, collect a deposit

from, seek compensation, remuneration or reimbursement from, or have any recourse against any MA Customer or person (other than MA Organization or an intermediary) acting on behalf of the MA Customer for Covered Services provided pursuant to the Agreement or for any other fees that are the legal obligation of MA Organization under the CMS Contract. This provision does not prohibit Provider from collecting from MA Customers allowable Cost Sharing. This provision also does not prohibit Provider and an MA Customer from agreeing to the provision of services solely at the expense of the MA Customer, as long as Provider has clearly informed the MA Customer, in accordance with applicable law, that the MA Customer's Benefit Plan may not cover or continue to cover a specific service or services.

In the event of MA Organization's or an intermediary's insolvency or other cessation of operations or termination of MA Organization's contract with CMS, Provider shall continue to provide Covered Services to an MA Customer through the later of the period for which premium has been paid to MA Organization on behalf of the MA Customer, or, in the case of MA Customers who are hospitalized as of such period or date, the MA Customer's discharge.

This provision shall be construed in favor of the MA Customer, shall survive the termination of the Agreement regardless of the reason for termination, including MA Organization's insolvency, and shall supersede any contrary agreement, oral or written, between Provider and an MA Customer or the representative of an MA Customer if the contrary agreement is inconsistent with this provision.

For the purpose of this provision, an "intermediary" is a person or entity authorized to negotiate and execute the Agreement on behalf of Provider or on behalf of a network through which Provider elects to participate.

3.4 Dual Eligible Customers. Provider agrees that in no event, including but not limited to, non-payment by a state Medicaid agency or other applicable regulatory authority, other state source, or breach by United of the Agreement, shall Provider bill, charge, collect a deposit from, seek compensation, remuneration or reimbursement from, or have any recourse against any Dual Eligible Customer, person acting on behalf of the Dual Eligible Customer, or MA Organization (unless notified otherwise) for Medicare Part A and B Cost Sharing. Instead, Provider will either: (a) accept payment made by or on behalf of MA Organization as payment in full; or (b) bill the appropriate state source for such Cost Sharing amount. If Provider imposes an excess charge on a Dual Eligible Customer, Provider is subject to any lawful sanction that may be imposed under Medicare or Medicaid. This provision does not prohibit Provider and a Dual Eligible Customer from agreeing to the provision of services solely at the expense of the Dual Eligible Customer, as long as Provider has clearly informed the Dual Eligible Customer, in accordance with applicable law, that the Dual Eligible Customer's Benefit Plan may not cover or continue to cover a specific service or services.

3.5 Eligibility. Provider agrees to immediately notify MA Organization in the event Provider is or becomes excluded from participation in any federal or state health care program under Section 1128 or 1128A of the Social Security Act. Provider also shall not employ or contract for the provision of health care services, utilization review, medical social work or administrative services, with or without compensation, with any individual or entity that has been excluded from

participation in any federal or state health care program under Section 1128 or 1128A of the Social Security Act.

3.6 **Laws.** Provider shall comply with all applicable federal and Medicare laws, regulations, and CMS instructions, including but not limited to: (a) federal laws and regulations designed to prevent or ameliorate fraud, waste, and abuse, including but not limited to, applicable provisions of federal criminal law, the False Claims Act (31 U.S.C. §3729 et seq.), and the anti-kickback statute (§1128B of the Social Security Act); and (b) HIPAA administrative simplification rules at 45 CFR Parts 160, 162, and 164.

3.7 **Federal Funds.** Provider acknowledges and agrees that MA Organization receives federal payments under the CMS Contract and that payments Provider receives from or on behalf of MA Organization are, in whole or in part, from federal funds. Provider is therefore subject to certain laws that are applicable to individuals and entities receiving federal funds.

3.8 **CMS Contract.** Provider shall perform the services set forth in the Agreement in a manner consistent with and in compliance with MA Organization's contractual obligations under the CMS Contract.

3.9 **Records.**

(a) Maintenance; Privacy and Confidentiality; Customer Access. Provider shall maintain records and information related to the services provided under the Agreement, including but not limited to MA Customer medical records and other health and enrollment information, in an accurate and timely manner. Provider shall maintain such records for at least ten (10) years or such longer period as required by law. Provider shall safeguard MA Customer privacy and confidentiality, including but not limited to the privacy and confidentiality of any information that identifies a particular MA Customer, and shall comply with all federal and state laws regarding confidentiality and disclosure of medical records or other health and enrollment information. Provider shall ensure that MA Customers have timely access to medical records and information that pertain to them, in accordance with applicable law.

(b) Government Access to Records. Provider acknowledges and agrees that the Secretary of Health and Human Services, the Comptroller General, or their designees shall have the right to audit, evaluate and inspect any pertinent books, contracts, medical records, patient care documentation and other records and information belonging to Provider that involve transactions related to the CMS Contract. This right shall extend through ten (10) years from the later of the final date of the CMS Contract period in effect at the time the records were created or the date of completion of any audit, or longer in certain instances described in the applicable Medicare Advantage regulations. For the purpose of conducting the above activities, Provider shall make available its premises, physical facilities and equipment, records relating to MA Customers, and any additional relevant information CMS may require.

(c) MA Organization Access to Records. Provider shall grant MA Organization or its designees such audit, evaluation, and inspection rights identified in subsection 3.9(b) as are necessary for MA Organization to comply with its obligations under the CMS Contract. Whenever possible, MA Organization will give Provider reasonable notice of the need for such audit, evaluation or inspection, and will conduct such audit, evaluation or inspection at a reasonable time and place. Provider shall submit medical records of MA Customers to the MA Organization as may be requested, within the timeframes specified, for the purpose of (i) CMS audits of risk adjustment data and (ii) for other purposes medical records from providers are used by MA Organization, as specified by CMS. Provision of medical records must be in the manner consistent with HIPAA privacy statute and regulations.

3.10 MA Organization Accountability; Delegated Activities. Provider acknowledges and agrees that MA Organization oversees and is accountable to CMS for any functions and responsibilities described in the CMS Contract and applicable Medicare Advantage regulations, including those that MA Organization may delegate to Provider or others. If MA Organization has delegated any of its functions and responsibilities under the CMS Contract to Provider pursuant to the Agreement, the following shall apply in addition to the other provisions of this Appendix:

- (a) Provider shall perform those delegated activities specified in the Agreement, if any, and shall comply with any reporting responsibilities as set forth in the Agreement.
- (b) If MA Organization has delegated to Provider any activities related to the credentialing of health care providers, Provider must comply with all applicable CMS requirements for credentialing, including but not limited to the requirement that the credentials of medical professionals must either be reviewed by MA Organization, or the credentialing process must be reviewed, pre-approved and audited on an ongoing basis by MA Organization.
- (c) If MA Organization has delegated to Provider the selection of health care providers to be participating providers in MA Organization's Medicare Advantage network, MA Organization retains the right to approve, suspend or terminate the participation status of such health care providers.
- (d) Provider acknowledges that MA Organization shall monitor Provider's performance of any delegated activities on an ongoing basis. If MA Organization or CMS determines that Provider has not performed satisfactorily, MA Organization may revoke any or all delegated activities and reporting requirements. Provider shall cooperate with MA Organization regarding the transition of any delegated activities or reporting requirements that have been revoked by MA Organization.

3.11 Subcontracts. If Provider has any arrangements, in accordance with the terms of the Agreement, with affiliates, subsidiaries, or any other subcontractors, directly or through another person or entity, to perform any of the services Provider is obligated to perform under the

Agreement that are the subject of this Appendix, Provider shall ensure that all such arrangements are in writing, duly executed, and include all the terms contained in this Appendix. Provider shall provide proof of such to MA Organization upon request. Provider further agrees to promptly amend its agreements with such subcontractors, in the manner requested by MA Organization, to meet any additional CMS requirements that may apply to the services.

3.12 **Offshoring.** Unless previously authorized by MA Organization in writing, all services provided pursuant to the Agreement that are subject to this Appendix must be performed within the United States, the District of Columbia, or the United States territories.

SECTION 4 OTHER

4.1 **Payment.** MA Organization or its designee shall promptly process and pay or deny Provider's claim no later than sixty (60) days after MA Organization or its designee receives all appropriate information as described in MA Organization's administrative procedures. If Provider is responsible for making payment to subcontracted providers for services provided to MA Customers, Provider shall pay them no later than sixty (60) days after Provider receives request for payment for those services from subcontracted providers.

4.2 **Regulatory Amendment.** MA Organization may unilaterally amend this Appendix to comply with applicable laws and regulations and the requirements of applicable regulatory authorities, including but not limited to CMS. MA Organization shall provide written or electronic notice to Provider of such amendment and its effective date. Unless such laws, regulations or regulatory authority(ies) direct otherwise, the signature of Provider will not be required in order for the amendment to take effect.

WASHINGTON STATE PROGRAMS REGULATORY REQUIREMENTS APPENDIX

PROVIDER

THIS WASHINGTON STATE PROGRAMS REGULATORY REQUIREMENTS APPENDIX (this “Appendix”) supplements and is made part of the provider agreement (the “Agreement”) between **UnitedHealthcare of Washington, Inc.** (referred to in this Appendix as “Carrier”) and the party named in the Agreement (“Provider”).

SECTION 1 APPLICABILITY

The requirements of this Appendix apply to State Program benefit plans sponsored, issued or administered by Carrier under the State’s Healthy Options and Basic Health programs (each, a “State Program” and collectively, the “State Programs” as further defined below) as governed by the State’s designated regulatory agencies. In the event of a conflict between this Appendix and other appendices or any provision of the Agreement, the provisions of this Appendix shall control except with regard to benefit plans outside the scope of this Appendix or unless otherwise required by law. In the event Carrier is required to amend or supplement this Appendix as required or requested by the State, Provider agrees that Carrier shall be permitted to unilaterally initiate such additions, deletions or modifications.

SECTION 2 DEFINITIONS

Unless otherwise defined in this Appendix, all capitalized terms shall be as defined in the Agreement. For purposes of this Appendix, the following terms shall have the meanings set forth below; provided, however, in the event any definition set forth in this Appendix or the Agreement is inconsistent with any definitions under the applicable State Program, the definitions shall have the meaning set forth under the applicable State Program.

2.1 Administrative Function(s): Any obligation of Carrier under a State Contract other than the direct provision of Covered Services to Covered Persons. Administrative Functions include, but are not limited to, utilization/medical management, claims processing, Covered Person grievances and appeals, and the provision of data or information necessary to fulfill any of Carrier’s obligations under a State Contract.

2.2 Basic Health: The Department's prepaid managed care health program for low income individuals.

2.3 Claim: A bill for services, a line item of service or all services for one Covered Person within a bill.

2.4 **Clean Claim:** A Claim that can be processed without obtaining additional information from the provider of the service or from a third party.

2.5 **Covered Person:** An individual who is currently enrolled with Carrier for the provision of services under a State Program. A Covered Person may also be referred to as an Enrollee, Member or Customer under the Agreement.

2.6 **Covered Services:** Health care services or products for which a Covered Person is enrolled with Carrier to receive coverage under a State Contract.

2.7 **Department:** The State of Washington Health Care Authority and its employees and authorized agents or another State of Washington entity responsible for the oversight and implementation of a State Program, as applicable.

2.8 **Healthy Options:** The Department's prepaid managed care health program for Medicaid-eligible persons and persons enrolled in the State Children's Health Insurance Program.

2.9 **Primary Care Provider or PCP:** A Provider who has responsibility for supervising, coordinating, and providing primary health care to Covered Persons, initiating referrals for specialist care, and maintaining the continuity of care for Covered Persons. PCPs include, but are not limited to, pediatricians, family practitioners, general practitioners, internists, physician assistants (under the supervision of a physician), or advanced registered nurse practitioners, as designated by Carrier. The definition of PCP is inclusive of primary care physician as it is used in 42 CFR § 438.

2.10 **Provider:** A hospital, ancillary provider, physician group, individual physician or other health care provider who has entered into an Agreement.

2.11 **State:** The State of Washington or its designated regulatory agencies.

2.12 **State Contract:** Carrier's contract with the Department for the purpose of providing and paying for Covered Services to Covered Persons enrolled in one or more State Programs.

2.13 **State Program(s):** The State of Washington Healthy Options and Basic Health programs. For purposes of this Appendix, State Program may refer to the State agency(ies) responsible for administering the applicable State Program.

SECTION 3 PROVIDER REQUIREMENTS

The State Programs, through contractual requirements and federal and State statutes and regulations, requires the Agreement to contain certain conditions that Carrier and Provider agree to undertake, which include the following:

3.1 Definitions Related to the Provision of Covered Services. Provider shall follow the applicable State Contract's requirements for the provision of Covered Services. Provider's decisions affecting the delivery of acute or chronic care services to Covered Persons shall be made on an individualized basis and in accordance with the following definitions:

(a) Emergency Medical Condition: A medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) such that a prudent layperson, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate medical attention to result in: (a) placing the health of the individual (or with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy; (b) serious impairment to bodily functions; or (c) serious dysfunction of any bodily organ or part.

(b) Emergency Services: Inpatient and outpatient contracted services furnished by a provider qualified to furnish the services needed to evaluate or stabilize an Emergency Medical Condition.

(c) Medically Necessary or Medical Necessity: Services that are "Medically Necessary" as defined in WAC 182-500-0070. In addition, Medically Necessary services shall include services related to a Covered Person's ability to achieve age-appropriate growth and development.

3.2 Medicaid Eligibility. If applicable, Provider must meet minimum requirements for participation in the State Programs. Provider may meet this requirement either by being enrolled with the State as a Medicaid provider or by demonstrating to Carrier that it meets the applicable minimum requirements for Medicaid participation. Carrier will exclude from its network any provider who has been suspended from the Medicare or Medicaid program in any state.

3.3 Primary Care Provider (PCP) Requirements. Providers who are PCPs shall comply with the PCP requirements of the State Contract, as set forth in the applicable provider manuals, protocols, policies and procedures that Carrier has provided or delivered to Provider.

3.4 Accessibility Standards. Provider shall provide for timely access for Covered Person appointments in accordance with the appointment availability requirements established under the State Contract, as further described in the applicable provider manual. Carrier shall regularly monitor Provider's compliance with timely access standards and Provider shall implement appropriate corrective action in the event Provider fails to comply with the appointment wait time requirements under the State Contract

3.5 Hours of Operation. Provider shall offer hours of operation that are no less than the hours of operation offered to commercial beneficiaries or comparable to Medicaid fee-for-service if Provider serves only Medicaid beneficiaries.

3.6 Hold Harmless. Except for any applicable cost-sharing requirements under the State Contract, Provider shall accept payment from Carrier as payment in full and shall not request payment from the Department or any Covered Person for Covered Services provided pursuant to

the Agreement and the State Contract. Provider shall hold the Department and its employees, the U.S. Department of Health and Human Services (DHHS) and Covered Persons harmless in the event that Carrier cannot or will not pay for such Covered Services. In accordance with 42 CFR Section 447.15, as may be amended from time to time, the Covered Person is not liable to Provider for any services for which Carrier is liable and as specified under the State's relevant health insurance or managed care statutes, rules or administrative agency guidance. Provider shall not require any copayment or cost sharing for Covered Services provided under the Agreement unless expressly permitted under the State Contract. Provider shall also be prohibited from charging Covered Persons for missed appointments if such practice is prohibited under the State Contract or applicable law. Neither the State, the Department nor Covered Persons shall be in any manner liable for the debts and obligations of Carrier and under no circumstances shall Carrier, or any providers used to deliver services covered under the terms of the State Contract, charge Covered Persons for Covered Services.

If the services are not Covered Services and, prior to providing the services, Provider informed the Covered Person that the services are not Covered Services. Provider shall inform the Covered Person of the non-Covered Service and have the Covered Person acknowledge the information. If the Covered Person still requests the service, Provider shall obtain such acknowledgment in writing prior to rendering the service and report to Carrier all instances where a Covered Person is charged for services. Carrier will determine whether a Covered Person was charged for services inappropriately and may recover such payment as applicable.

This provision shall survive any termination of the Agreement, including breach of the Agreement due to insolvency.

3.7 Indemnification. To the extent applicable to Provider in performance of the Agreement, Provider shall indemnify, defend and hold the Department and its employees harmless from and against all injuries, deaths, losses, damages, claims, suits, liabilities, judgments, costs and expenses, including court costs and attorney fees, to the extent proximately caused by any negligent act or other intentional misconduct or omission of Provider, its agents, officers, employees or contractors arising from the Agreement. The Department may waive this requirement for public entities if Provider is a state agency or sub-unit as defined by the State or a public health entity with statutory immunity. This clause shall survive the termination of the Agreement for any reason, including breach due to insolvency.

3.8 Provider Selection. To the extent applicable to Provider in performance of the Agreement, Provider shall comply with 42 CFR 438.214, as may be amended from time to time, which includes, but is not limited to the selection and retention of providers, credentialing and recredentialing requirements and nondiscrimination. If Carrier delegates credentialing to Provider, Carrier will provide monitoring and oversight and Provider shall ensure that all licensed medical professionals are credentialed in accordance with Carrier's and the State Contract's credentialing requirements.

3.9 Restrictions on Referrals. Provider shall not make inappropriate referrals for designated health services to health care entities with which Provider or a member of Provider's family has

a financial relationship, pursuant to federal anti-kickback and physician self-referral laws that prohibit such referrals.

3.10 Subcontracts. Provider shall perform those services set forth in the Agreement, and may subcontract services only if permitted by Carrier in writing. If Provider subcontracts or delegates any functions of the Agreement, in accordance with the terms of the Agreement, the subcontract or delegation must include all of the requirements of this Appendix, and applicable requirements of the State Contract. Provider further agrees to promptly amend its agreements with such subcontractors, in the manner requested by Carrier, to meet any additional State Program requirements that may apply to the services.

3.11 Records Retention. As required under State or federal law or the State Contract, Provider shall maintain an adequate record keeping system for recording services, charges, dates and all other commonly accepted information elements for services rendered pursuant to the State Contract. All financial records shall follow generally accepted accounting principles. Medical records and supporting management systems shall include all pertinent information related to the medical management of each Covered Person. Other records shall be maintained as necessary to clearly reflect all actions taken by Provider related to services provided under the State Contract. Records shall be maintained for a period of not less than six (6) years from the close of the State Contract, or such other period as required by law. If records are under review or audit, they must be retained for a minimum of six (6) years following resolution of such action. Prior approval for the disposition of records must be requested and approved by Carrier if the Agreement is continuous.

3.12 Records Access. Provider acknowledges and agrees that the State, the U.S. Department of Health and Human Services and other authorized federal and state personnel shall have the right to evaluate through audit, inspection or other means, any records pertinent to the State Contract, including records pertaining to the quality, appropriateness and timeliness of services performed under the State Contract. This clause shall survive the termination of the Agreement for any reason, including breach due to insolvency.

3.13 Government Audit; Investigations. Provider acknowledges and agrees that the State and the U.S. Department of Health and Human Services or their authorized representatives shall have the right to inspect or otherwise evaluate the quality, appropriateness, and timeliness of services provided under the terms of the State Contract and any other applicable rules. There shall be no restrictions on the right of the State or federal government to conduct whatever inspections and audits are necessary to assure quality, appropriateness or timeliness of services provided pursuant to the State Contract and the reasonableness of their costs. Provider shall provide immediate access to facilities, records and supportive materials pertinent to the State Contract for State or Federal fraud investigators.

3.14 Privacy; HIPAA. Provider shall comply with all applicable privacy rule and security rule provisions of the Health Insurance Portability and Accountability Act of 1996 ("HIPAA"), and associated implementing regulations, as may be amended from time to time, and shall safeguard information about Covered Persons in accordance with applicable federal and State

privacy laws and rules including 42 CFR §438.224 and 42 CFR Part 431, Subpart F, as may be amended from time to time.

3.15 Compliance with Law. Provider shall comply with all applicable federal and State laws and regulations, including but not limited to the following to the extent applicable to Provider in performance of the Agreement:

- (a) Title XIX and Title XXI of the Social Security Act;
- (b) Title VI of the Civil Rights Act of 1964;
- (c) Title IX of the Education Amendments of 1972, regarding any education programs and activities;
- (d) The Age Discrimination Act of 1975;
- (e) The Rehabilitation Act of 1973;
- (f) The Budget Deficit Reduction Act of 2005;
- (g) The False Claims Act;
- (h) The Health Insurance Portability and Accountability Act (HIPAA);
- (i) The American Recovery and Reinvestment Act (ARRA);
- (j) The Patient Protection and Affordable Care Act (PPACA or ACA);
- (k) Health Care and Education Reconciliation Act;
- (l) Chapter 70.02 RCW and the Washington State Patient Bill of Rights, including, but not limited to, the administrative and financial responsibility for independent reviews;
- (m) All federal and State professional and facility licensing and accreditation requirements/standards that apply to the services Provider performs pursuant to the Agreement, including but not limited to:
 - (i) All applicable standards, orders or requirements issued under Section 306 of the Clean Water Act (33 USC 1368), Executive Order 11738, and Environmental Protection Agency (EPA) regulations (40 CFR Part 15), which prohibit the use of facilities included on the EPA List of violating Facilities. Any violations must be reported to the Department, DHHS, and the EPA;
 - (ii) Any applicable mandatory standards and policies relating to energy efficiency that are contained in the State Energy Conservation Plan, issued in compliance with the Federal Energy Policy and Conservation Act.
 - (iii) Those specified for laboratory services in the clinical Laboratory Improvement Amendments (CLIA);
 - (iv) Those specified in Title 18 RCW for professional licensing;
 - (v) Industrial Insurance – Title 51 RCW;
 - (vi) Reporting of abuse as required by RCW 26.44.030;
 - (vii) Federal Drug and Alcohol Confidentiality Laws in 42 CFR Part 2;
 - (viii) Equal Opportunity in Employment (EEO) provisions, including but not limited to compliance with E.O. 11246, “Equal Employment Opportunity,” as amended by E.O. 11375, “Amending Executive Order 11246 Relating to Equal Employment Opportunity,” and as supplemented by regulations at 41 CFR part 60, “Office of Federal Contract Compliance Programs, Equal Employment Opportunity, Department of Labor.”

- (ix) The Copeland Anti-Kickback Act;
- (x) The Davis-Bacon Act;
- (xi) The Byrd anti-Lobbying Amendment;
- (xii) All federal and State nondiscrimination laws and regulations;
- (xiii) The American with Disabilities Act (ADA). Provider shall make reasonable accommodation for Covered Persons with disabilities in accord with the ADA for all Covered Services and shall assure physical and communication barriers do not inhibit Covered Persons with disabilities from obtaining Covered Services;
- (xiv) Any other requirements associated with the receipt of federal funds.

(n) Applicable State and federal rules and regulations as set forth in the State Contract, including the applicable requirements of 42 CFR 438.6(i).

3.16 Physician Incentive Plans. In the event Provider participates in a physician incentive plan ("PIP") under the Agreement, Provider agrees that such PIPs must comply with 42 CFR 417.479, 42 CFR 438.6(h), 42 CFR 422.208, and 42 CFR 422.210, as may be amended from time to time. Neither Carrier nor Provider may make a specific payment directly or indirectly under a PIP to a physician or physician group as an inducement to reduce or limit Medically Necessary services furnished to an individual Covered Person. PIPs must not contain provisions that provide incentives, monetary or otherwise, for the withholding of Medically Necessary care.

3.17 Lobbying. Provider agrees to comply with the following requirements related to lobbying:

(a) Prohibition on Use of Federal Funds for Lobbying: By signing the Agreement, Provider certifies to the best of Provider's knowledge and belief, pursuant to 31 U.S.C. Section 1352 and 45 CFR Part 93, as may be amended from time to time, that no federally appropriated funds have been paid or will be paid to any person by or on Provider's behalf for the purpose of influencing or attempting to influence an officer or employee of any agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with the award of any federal contract, the making of any federal grant, the making of any federal loan, the entering into of any cooperative agreement, or the extension, continuation, renewal, amendment, or modification of any federal contract, grant, loan, or cooperative agreement.

(b) Disclosure Form to Report Lobbying: If any funds other than federally appropriated funds have been paid or will be paid to any person for the purpose of influencing or attempting to influence an officer or employee of any agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with the award of any federal contract, the making of any federal grant, the making of any federal loan, the entering into of any cooperative agreement, or the extension, continuation, renewal, amendment or modification of any federal contract, grant, loan, or cooperative agreement and the value of the Agreement exceeds \$100,000, Provider shall complete and submit Standard Form-LLL, "Disclosure Form to Report Lobbying," in accordance with its instructions.

3.18 Excluded Individuals and Entities. By signing the Agreement, Provider certifies to the best of Provider's knowledge and belief that neither it nor any of its principals, nor any providers, subcontractors or consultants with whom Provider contracts and who are providing items or services that are significant and material to Provider's obligations under the Agreement is:

- (a) excluded from participation in federal health care programs under either Section 1128 or section 1128A of the Social Security Act; or
- (b) debarred, suspended or otherwise excluded from participating in procurement activities under the Federal Acquisition Regulation or from participating in nonprocurement activities under regulations issued under Executive Order no. 12549 or under guidelines implementing Executive Order No. 12549; or an affiliate, as defined in the Federal Acquisition Regulation, of such a person.

Provider is obligated to screen its employees and contractors to determine whether any of them have been excluded from participation in Medicare, Medicaid, CHIP, or any Federal Health Care Programs (as defined in Section 1128B(f) of the Social Security Act). Provider shall not employ or contract with an individual or entity that has been excluded. Provider shall immediately report to Carrier any exclusion information discovered. Provider acknowledges and agrees that civil monetary penalties may be imposed against Provider if he or she employs or enters into contracts with excluded individuals or entities to provide items or Covered Services. Provider can search the HHS-OIG website, at no cost, by the names of any individuals or entities. The database is called LEIE and can be accessed at <http://www.oig.hhs.gov/fraud/exclusions.asp>. Carrier will terminate the Agreement immediately and exclude from its network any provider who has been excluded from the Medicare or Medicaid program in any state.

3.19 Disclosure. Provider shall cooperate with Carrier in disclosing information the Department may require related to ownership and control, significant business transactions, and persons convicted of crimes in accordance with 42 C.F.R. §§ 455.104, 455.105, and 455.106.

3.20 Cultural Competency. Provider shall participate in Carrier's and the State's efforts to promote the delivery of services in a culturally competent manner to all Covered Persons, including those with limited English proficiency and diverse cultural and ethnic backgrounds. Provider shall provide information to Covered Persons regarding treatment options and alternatives in a manner appropriate to the Covered Person's condition and ability to understand.

3.21 Marketing. As required under State or federal law and the State Contract, any marketing materials developed and distributed by Provider as related to the performance of the Agreement must be submitted to Carrier to submit to the State Program for prior approval.

3.22 Fraud, Waste and Abuse Prevention. Provider shall cooperate fully with Carrier's policies and procedures designed to protect program integrity and prevent and detect potential or suspected fraud, waste, and abuse in the administration and delivery of services under the State contract and shall cooperate and assist the State Program and any other State or federal agency

charged with the duty of preventing, identifying, investigating, sanctioning or prosecuting suspected fraud, waste, and abuse in state and/or federal health care programs.

In accordance with Carrier's policies and the Deficit Reduction Act of 2005 (DRA), Provider shall have written policies for its employees, contractors or agents that: (a) provide detailed information about the federal False Claims Act (established under sections 3729 through 3733 of title 31, United States Code); (b) cite administrative remedies for false claims and statements (established under chapter 38 of title 31, United States Code) and whistleblower protections under federal and state laws; (c) reference state laws pertaining to civil or criminal penalties for false claims and statements; and (d) with respect to the role of such laws in preventing and detecting fraud, waste, and abuse in federal health care programs (as defined in section 1128B(f)), include as part of such written policies, detailed provisions regarding Provider's policies and procedures for detecting and preventing fraud, waste, and abuse. Provider agrees to train its staff on the aforesaid policies and procedures.

3.23 Data; Reports. Provider shall cooperate with and release to Carrier any information necessary for Carrier to perform its obligations under the State Contract, including the timely submission of reports and information required by Carrier. Such reports shall include child health check-up reporting, if applicable, as well as complete and accurate encounter data in accordance with the requirements of Carrier and the State Contract.

3.24 Insurance Requirements. As applicable, Provider shall secure and maintain during the term of the Agreement insurance appropriate to the services to be performed under the Agreement. Upon request, Provider shall make available to Carrier copies of its Certificate(s) of Insurance.

3.25 Quality; Utilization Management. Pursuant to any applicable provider manuals and related protocols, or as elsewhere specified under the Agreement, Provider agrees to cooperate with Carrier's quality assessment, performance improvement and utilization review and management activities, which shall be tailored to the nature and type of services subcontracted. This shall include, but not be limited to, participation in any internal and external quality assurance, utilization review, peer review, and grievance procedures established by Carrier or as required under the State Contract to ensure quality control for the health care provided, including but not limited to the accessibility of medically necessary health care, and Covered Persons have due process for their complaints, grievances, appeals, fair hearings or requests for external review of adverse decisions made by Carrier or Provider. Provider shall adhere to the quality assurance and utilization review standards of the State Program and shall monitor quality and initiate corrective action to improve quality consistent with the generally accepted level of care.

3.26 Continuity of Care. Provider shall cooperate with Carrier and provide a Covered Person with continuity of treatment, including coordination of care to the extent required under law, in the event Provider's participation with Carrier terminates during the course of a Covered Person's treatment by Provider.

3.27 Advance Directives. Providers that are hospitals, nursing facilities, home health agencies, hospices, or organizations responsible for providing personal care, as well as PCPs that

contract with any of the above entities, shall comply with federal and State law regarding advance directives for adult Covered Persons.

3.28 **Termination.** In the event of termination of the Agreement, Provider shall promptly supply to Carrier all information necessary for the reimbursement of any outstanding Medicaid claims.

3.29 **Health Information Systems.** Provider shall maintain a health information system that complies with the requirements of 42 CFR 438.242 and provides the information necessary to meet Provider's obligations under the Agreement and this Appendix.

SECTION 4 ADDITIONAL REQUIREMENTS FOR DELEGATED ADMINISTRATIVE FUNCTIONS

4.1 This Section applies to those Providers to whom Carrier has delegated an Administrative Function. Provider shall perform those delegated Administrative Functions set forth in the Agreement through an exhibit or otherwise. Any changes or modifications to the Administrative Functions shall be agreed to in writing by the parties.

4.2 Prior to delegation, Carrier shall perform an evaluation of Provider's ability to successfully perform and meet the requirements of the State Contract for any delegated Administrative Function.

4.3 Provider agrees to cooperate with Carrier's requirements for delegation of Administrative Functions, including but not limited to ongoing monitoring and an annual evaluation for the purpose of determining Provider's compliance with requirements related to the delegated Administrative Functions. As a result of such monitoring activities, Carrier shall identify to Provider any deficiencies or areas for improvement mandated under the applicable State Contract and Provider shall take appropriate corrective action.

4.4 If Provider is at financial risk, Provider shall comply with, and maintain throughout the term of the Agreement, the solvency requirements established by Carrier from time to time. Carrier shall monitor Provider's compliance with such solvency requirements.

4.5 Provider shall maintain records necessary to adequately document the performance of delegated Administrative Functions.

4.6 In addition to its termination rights under the Agreement, Carrier shall have the right to revoke any delegated Administrative Functions Carrier delegates to Provider under the Agreement or impose other sanctions consistent with the applicable State Contract if in Carrier's reasonable judgment Provider's performance of a delegated Administrative Function is inadequate.

SECTION 5 CARRIER REQUIREMENTS

5.1 Prompt Payment. Carrier shall pay Provider pursuant to the State Contract, applicable State law and regulations, including the timeliness of payment standards specified for health carriers in WAC 284-43-521 and the timeliness of payment standards specified for Medicaid fee-for-service in Section 1902(a)(37)(A) of the Social Security Act and 42 CFR 447.46, 42 CFR 447.45(d)(2), 42 CFR 447.45(d)(3), 42 CFR 447.45(d)(5) and 42 CFR 447.45(d)(6), as may be amended from time to time. To be compliant with both payment standards, Carrier shall pay or deny ninety-five percent (95%) of Clean Claims within thirty (30) calendar days of receipt, ninety-five percent (95%) of all Claims within sixty (60) calendar days of receipt and ninety-nine percent (99%) of Clean Claims within ninety (90) calendar days of receipt; provided, however, that Carrier and Provider may agree to a different payment requirement in writing on an individual Claim. If a third party liability exists, payment of Claims shall be determined in accordance with federal and/or State third party liability law and the terms of the applicable State Contract. Unless Carrier otherwise requests assistance from Provider, Carrier will be responsible for third party collections in accordance with the terms of the applicable State Contract.

5.2 No Incentives to Limit Medically Necessary Services. Carrier shall not structure compensation provided to individuals or entities that conduct utilization management and concurrent review activities so as to provide incentives for the individual or entity to deny, limit, or discontinue Medically Necessary services to any Covered Person.

5.3 Provider Discrimination Prohibition. In accordance with 42 CFR 438.12 and 438.214(c), Carrier shall not discriminate with respect to the participation, reimbursement or indemnification of a provider who is acting within the scope of such provider's license or certification under applicable State law, solely on the basis of such license or certification. Further, Carrier shall not discriminate with respect to the participation, reimbursement or indemnification of any provider who serves high-risk Covered Persons or specializes in conditions requiring costly treatments. This provision shall not be construed as prohibiting Carrier from limiting a provider's participation to the extent necessary to meet the needs of Covered Persons. This provision also is not intended and shall not interfere with measures established by Carrier that are designed to maintain quality of care practice standards and control costs.

5.4 Communications with Covered Persons. Carrier shall not prohibit or otherwise restrict Provider, when acting within the lawful scope of practice, from advising or advocating on behalf of a Covered Person for the following:

- (a) The Covered Person's health status, medical care, or treatment options, including any alternative treatment that may be self-administered;
- (b) Any information the Covered Person needs in order to decide among all relevant treatment options;

- (c) The risks, benefits, and consequences of treatment or non-treatment; or
- (d) The Covered Person's right to participate in decisions regarding his or her health care, including the right to refuse treatment, and to express preferences about future treatment decisions.

Carrier also shall not prohibit a Provider from advocating on behalf of a Covered Person in any grievance system, utilization review process, or individual authorization process to obtain necessary health care services.

5.5 Grievance & Appeals. Carrier will supply Provider with information regarding Carrier's grievance and appeals system, including: (a) the toll-free numbers to file oral grievances and appeals; (b) the availability of assistance in filing a grievance or appeal; (c) a Covered Person's rights to request continuation of benefits during an appeal or hearing and, if Carrier's action is upheld, the Covered Person's responsibility to pay for the continued benefits; (d) a Covered Person's right to file grievances and appeals and the requirements and timeframes for filing; and (e) a Covered Person's right to a hearing, how to obtain a hearing, and representation rules at a hearing.

5.6 Revocation of Delegated Activities. In addition to Carrier's termination rights under the Agreement, Carrier shall have the right to revoke any functions or activities Carrier delegates to Provider under the Agreement or impose other sanctions consistent with the State Contract if in Carrier's reasonable judgment Provider's performance under the Agreement is inadequate. Carrier shall also have the right to suspend, deny, refuse to renew or terminate Provider in accordance with the terms of the State Contract and applicable law and regulation.

SECTION 6 OTHER REQUIREMENTS

6.1 Compliance with State Contract. All tasks performed under the Agreement shall be performed in accordance with the requirements of the State Contract, as set forth in this Appendix, applicable provider manuals, and protocols, policies and procedures that Carrier has provided or delivered to Provider. The applicable provisions of the State Contract are incorporated into the Agreement by reference. Nothing in the Agreement relieves Carrier of its responsibility under the State Contract. If any provision of the Agreement is in conflict with provisions of the State Contract, the terms of the State Contract shall control and the terms of the Agreement in conflict with those of the State Contract will be considered waived.

6.2 Monitoring. Carrier shall perform ongoing monitoring (announced or unannounced) of services rendered by Provider under the Agreement and shall perform periodic formal reviews of Provider according to a schedule established by the State, consistent with industry standards or State managed care organization laws and regulations or requirements under the State Contract. As a result of such monitoring activities, Carrier shall identify to Provider any deficiencies or areas for improvement mandated under the State Contract and Provider and Carrier shall take appropriate corrective action. Provider shall comply with any corrective action plan initiated by Carrier and/or required by the State Program. In addition, Provider shall monitor and report the quality of services delivered under the Agreement and initiate a plan of correction where

necessary to improve quality of care, in accordance with that level of care which is recognized as acceptable professional practice in the respective community in which Carrier and Provider practice and/or the performance standards established under the State Contract.

6.3 **Assignment.** No assignment of the Agreement shall take effect without the written agreement of the Department.

6.4 **Termination Notice.** Notwithstanding the termination provisions set forth in the Agreement, Provider and Carrier shall provide at least ninety (90) days advance notice to the other party of intent to terminate the Agreement without cause. Such notice shall be in accordance with the terms of the Agreement.